M.PHIL. CLINICAL PSYCHOLOGY · COMPREHENSIVE STUDY NOTES

Psychotherapy — Unit I: Introduction to Psychotherapy

An exceptionally detailed, source-grounded, exam-oriented revision resource integrating multiple standard psychotherapy texts.

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TOPIC A

1 · Definitions of Psychotherapy

What psychotherapy *is* — the problem of definition, the canonical working definition, and how different traditions circumscribe the term.

1.1 Introduction

Psychotherapy *(literally "treatment of the mind/soul"; broadly, the treatment of psychological problems by psychological means)* is one of the most ambiguous terms in the mental-health lexicon. Everyday speech blends it with helping, treating, advising, guiding, educating and influencing, and different schools attach different meanings. Wolberg observes that the term is genuinely slippery and that published definitions agree on only one point — that psychotherapy addresses *problems of an emotional nature* — while disagreeing on almost everything else. **(Wolberg-Technique, p.29; p.41)**

For the trainee, a precise definition matters because it *circumscribes the boundaries of operation* — it tells us who may practise, on whom, by what means, and towards what end — which in turn makes the activity teachable, regulable and researchable. A definition is therefore not a pedantic exercise: it is the hinge on which training standards (Topic 3), ethics and law (Topic 4), rights and responsibilities (Topic 5) and the whole evidence base (Topic 10) all turn.

Etymology and historical origin

The word itself tells a story. "Psychotherapy means *caring for another person's soul or being*, as derived from the Greek ***psyche*** *(soul or being)* and ***therapeutikos*** *(caring for another)*." **(Notes-Psychotherapies, p.0)** *(Kleinke, 1994, cited in Notes-Psychotherapies.)* The earliest and most famous informal description is ***"the talking cure"*** *(the phrase coined by Anna O., Breuer's early psychoanalytic patient, to capture that talking through and verbalising one's pain can itself be healing)*. It is "an elegant, albeit vague description" that "does not satisfy contemporary

psychotherapy researchers, but ... does provide an historical and foundational frame." (Notes-Psychotherapies, p.0) Modern practice, by contrast, "combines the synergistic power of a wealth of **biological, psychological, and social** resources ... requiring a great deal of creativity, competence, and collaboration." (Notes-Psychotherapies, p.0)

Psychotherapy as a *unilateral* professional relationship

A defining structural feature, often examined, is that psychotherapy is "a ***unilateral professional relationship*** *(a deliberately 'one-way' relationship in which the patient's problems — not the therapist's needs — are the sole focus)* ... circumscribed by limits on **time, frequency of contact, content of discussions, and level of intimacy**." (Notes-Psychotherapies, p.1) This is why "a psychologist will not be effective if he or she tries to be a friend, a colleague, or a lover," and why ***confidentiality*** — with explicit "ethical and legal limits ... including serious and immediate danger to self and others" — and the prohibition on ***dual relationships*** are built into the very concept of what psychotherapy *is*, not bolted on afterwards. (Notes-Psychotherapies, p.1) These limits anticipate Topics 4 and 5 directly.

1.2 Definitions (identify source / author)

CANONICAL WORKING DEFINITION — WOLBERG

"Psychotherapy is the **treatment, by psychological means, of problems of an emotional nature** in which a trained person deliberately establishes a professional relationship with the patient with the object of **(1) removing, modifying, or retarding existing symptoms, (2) mediating disturbed patterns of behavior,** and **(3) promoting positive personality growth and development.**" (Wolberg-Technique, p.29)

Wolberg deconstructs the definition clause by clause: ***"treatment"*** *(it is a form of treatment, not merely re-education, helping or guidance — those words only describe what happens inside it)*; and ***"psychological means"*** *(a generic umbrella covering the whole spectrum of psychological*

methods, e.g. interpretation, support, behavioural techniques).
(Wolberg-Technique, p.29)

THE SCEPTICAL "DEFINITION" — BOULDER CONFERENCE (1949)

Corsini quotes the wry Boulder-era description of psychotherapy as "*an undefined technique which is applied to unspecified problems with a nonpredictable outcome. For this technique we recommend rigorous training*" (Lehner, 1952). Corsini stresses there is still "no general agreement about the definition, methodology, or value" of evidence-based psychotherapy. The ***Boulder Conference*** *(the 1949 conference that established the scientist-practitioner model of clinical-psychology training)* arose from the APA's Shakow committee, appointed under then-president Carl Rogers.
(Corsini, p.592)

CONTEMPORARY / APA-ADOPTED DEFINITION — NORCROSS (1990)

Psychotherapy is "the **informed and intentional application of clinical methods and interpersonal stances derived from established psychological principles** for the purpose of assisting people to modify their behaviors, cognitions, emotions, and/or other personal characteristics in directions that the participants deem desirable."
(Notes-Psychotherapies, p.0–1; APA, 2012 Resolution) *(Norcross, 1990, p. 218, cited in Notes-Psychotherapies and adopted in the APA's 2012 Resolution on the Recognition of Psychotherapy Effectiveness.)*

The ***American Psychological Association (APA)*** *(the major US professional body for psychology)* describes psychotherapy, in general terms, as any psychological service provided by a trained professional that primarily uses *communication and interaction* to assess, diagnose and treat dysfunctional emotional reactions, ways of thinking and behaviour patterns. (APA Dictionary; verify against live entry) Note how this

echoes Wolberg's three clauses: it names a *trained professional*, a *relationship of communication*, and *defined aims*.

A spectrum of historical definitions (identify the author)

The Notes list a telling spectrum, running from the playful to the near-scientific, each attributable to a named author — a frequent exam gift, because quoting two or three with attribution signals breadth: **(Notes-Psychotherapies, p.0; all authors cited therein)**

1. "A **conversation with a therapeutic purpose**" (Korchin, 1976).
2. "The **purchase of friendship**" (Schofield, 1964) — a deliberately provocative framing of the paid, bounded relationship.
3. Two people interacting "with the specific goal of accomplishing something **beneficial for the complaining person**" (Bruch, 1981).
4. "When one person with an emotional disorder gets help from another person who has **less of an emotional disorder**" (J. Watkins, 1983).
5. Wolberg (1995) — the operational, clause-by-clause definition reproduced above.

Other framings observed across sources

- **Problem-centred (Wolberg-506Q).** The recurring "general aspects" of psychotherapy are its basic ingredients, the factors producing therapeutic change, the ***therapeutic alliance*** *(the collaborative, affective working bond between therapist and client)*, insight, resistance and termination. **(Wolberg-506Q, p.16)**
- **Person-centred / anti-reductionist (McWilliams).** Treatment "ought to flow from a comprehensive understanding of the client" rather than fitting patients "à la Procrustes" to a fixed technique — a caution against defining therapy as a single method. **(McWilliams, p.8)**
- **Common-factors framing.** Beyond any single definition, the Notes identify **four common factors** present whenever psychotherapy helps: (1) a relationship carrying *confidence* that the therapist is competent and caring; (2) a shared *expectancy/hope* of benefit; (3) an underlying *conceptual framework / rationale* with

procedures requiring the active participation of both; and (4) a *socially sanctioned healing setting*. On this view, psychotherapy is partly defined by the common ingredients that make any helping conversation curative. "Generally 70% to 80% of those treated in psychotherapy are improved (Smith et al., 1980)." (Notes-Psychotherapies, p.1) *(Smith et al., 1980, cited in Notes-Psychotherapies.)*

1.3 Core concepts within the definition

- **A trained person.** Competence and training are built into the definition (hence Topic 3). An untrained listener giving advice is not doing psychotherapy.
- **Deliberate, professional relationship.** Distinguishes therapy from friendship or spontaneous emotional support; the relationship is purposeful, bounded and ethically governed (Topics 4–5).
- **Psychological means.** Change is sought through psychological processes (learning, relationship, insight, emotional processing) rather than primarily physical means such as medication or surgery.
- **Problems of an emotional nature.** The target is psychological distress, maladaptive patterns and impaired functioning.
- **Defined objectives.** Symptom change, behavioural change and personality growth (expanded in Topic 2).

1.4 Theoretical foundations: why traditions define it differently

The definitional disagreement maps onto the major theoretical families. The CBT/empirical tradition (Wolberg's operational stance) prefers a precise, bounded definition so the activity can be measured and studied; the psychodynamic/humanistic voice resists reducing a deeply personal, relational process to a replicable "technique." (Wolberg-Technique, p.29; McWilliams, p.8) This is a classic exam contrast: *operational definition vs. understanding-based practice*.

The Notes name **four foundational schools**, each of which implicitly defines psychotherapy by what it treats as the agent of change: (Notes-Psychotherapies, p.2)

- **Psychoanalytic / psychodynamic** — change comes through ***insight*** and the relationship itself, offering a ***"corrective emotional experience"*** *(re-experiencing an old conflict with a new, healthier outcome in the therapy relationship)*; attends to ***transference*** *(the client's feelings redirected onto the therapist)* and ***countertransference*** *(the therapist's own reactions to the client)*. (Notes-Psychotherapies, p.2)
- **Behavioural / cognitive-behavioural (CBT)** — change comes through *new learning*; CBT "places an emphasis on analyzing current problems, defining **concrete goals**, and developing strategies so that the client can achieve specific, measurable changes." (Notes-Psychotherapies, p.3)
- **Humanistic / experiential** — change comes through *growth, acceptance and the actualising tendency* within an authentic, empathic relationship.
- **Family systems** — change comes through *altering patterns of relating* within the family or wider system.

Table 1.0 — How three traditions define the agent of change (exam contrast)

Dimension	Cognitive-behavioural (CBT)	Psychodynamic	Humanistic
View of the problem	Maladaptive learning, thoughts & behaviours	Unconscious conflict, early relational patterns	Blocked growth; incongruence; conditions of worth
Agent of change	New learning; skills; cognitive restructuring	Insight + corrective emotional experience (Notes-Psychotherapies, p.2)	Empathy, acceptance, the actualising tendency
Definition of therapy preferred	Operational, measurable, concrete goals (Notes-Psychotherapies, p.3)	Understanding-based; resists manualisation (McWilliams, p.8)	Relationship-based; person > technique
Typical stance on definition	Tight, testable	Broad, process-focused	Experiential, non-reductive

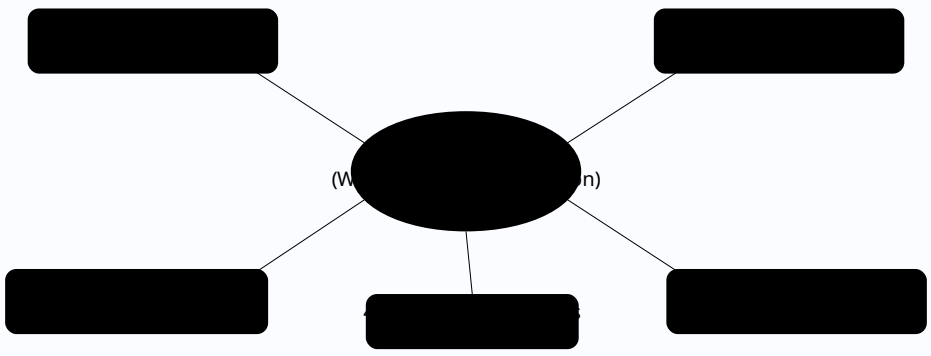


Figure 1.1 — The five core elements of the definition. *A concept map of Wolberg's working definition: a trained person in a professional relationship uses psychological means to treat emotional problems toward defined objectives (symptom change, pattern change, growth).*

1.5 Comparison table — psychotherapy vs. related interventions

A frequent exam demand is to distinguish psychotherapy from counselling, psychological assessment and psychiatric (biological) treatment. The table below summarises typical distinctions; note these are *characteristic tendencies*, not rigid walls — the activities overlap heavily in real practice.

Table 1.1 — Psychotherapy compared with related interventions (characteristic tendencies)

Dimension	Psychotherapy	Counselling	Psychological assessment	Psychia (biolog treatm
Primary aim	Modify symptoms, maladaptive patterns & promote growth (Wolberg-Technique, p.29)	Support adjustment, problem-solving & decision-making around specific life issues	Describe, measure & diagnose psychological functioning (inform the plan)	Alter br physiol reduce sympto medica
Typical focus	Deeper/enduring patterns; emotional & relational change	Present, situational concerns; everyday problems in living	Current functioning, traits, cognition, symptom severity	Neuroc physiol targets
Method	Psychological means within a professional relationship	Psychological means; often shorter, more guidance-oriented	Standardised tests, interview, observation	Pharma ECT an somati metho
Core practitioner	Trained psychotherapist / clinical psychologist	Counsellor / counselling psychologist	Clinical psychologist / assessor	Psychia (medica
Relationship to therapy	—	Overlaps; the line is "ambiguous" and blurred (Wolberg-Technique, p.29)	Often a *pre/post* step feeding the plan (see Topic 9)	Freque *combin* psycho

CAVEAT

Wolberg explicitly notes the term's ambiguity and overlap with helping, advising and guiding. Treat the table as a teaching heuristic; many authors use "counselling and psychotherapy" interchangeably (e.g. Corey's title). The Notes draw one clean line worth memorising: psychotherapy implies the patient is *treated to be cured/cared for*, whereas ***counselling*** is closer to "**providing advice and suggestions**."

(Wolberg-Technique, p.29; Notes-Psychotherapies, p.1; Corey)

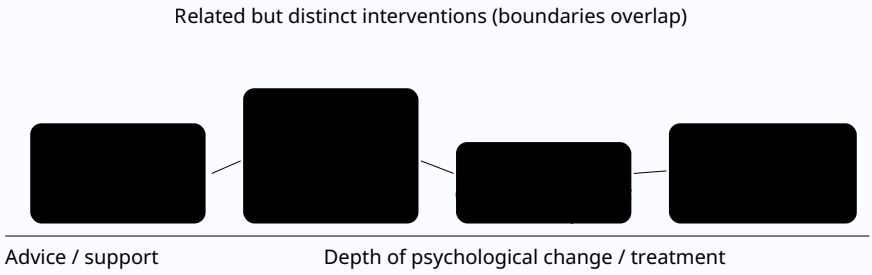


Figure 1.2 — Psychotherapy vs counselling vs assessment vs psychiatric treatment. *Characteristic tendencies placed on a depth-of-change axis; dashed arrows mark the real-world overlap Wolberg warns about. Assessment typically feeds therapy (Topic 9); psychiatric treatment is frequently combined with it.*

1.6 Clinical application

ILLUSTRATIVE EXAMPLE

Illustrative teaching vignette. A trainee receives a referral for "a 28-year-old with panic attacks." Applying the definition operationally, the trainee asks: is a *trained* person forming a *deliberate professional relationship*, using *psychological means*, targeting *emotional problems*, with *defined objectives*? If medication alone were offered with no psychological work, it would be psychiatric treatment, not psychotherapy; if a friend simply offered reassurance, it would be neither.

ILLUSTRATIVE EXAMPLE — THE "UNILATERAL RELATIONSHIP" TEST

Illustrative teaching vignette. A client asks the therapist out for coffee "as friends." The therapist uses the *unilateral professional relationship* criterion: the relationship is bounded in time, frequency, content and intimacy, and the patient's — not the therapist's — concerns are the focus. Accepting the invitation would blur it into a **dual relationship** and compromise the very thing that defines the work. The therapist warmly declines and explores the wish in session. (Notes-Psychotherapies, p.1)

1.7 Mnemonic

TRIP-E DEFINITION

- **T** **Trained** person delivers it (competence is built in).
- **R** **Relationship** — a deliberate, professional therapeutic relationship.
- **I** **Intentional objectives** — remove/modify symptoms, change patterns, promote growth.
- **P** **Psychological means** (not primarily physical/biological).
- **E** **Emotional problems** are the target.

Apply: *Asked "Is X psychotherapy?", run TRIP-E — a GP prescribing an antidepressant in a 5-minute visit fails "Psychological means" and "Relationship," so it is treatment but not psychotherapy.*

1.8 Advantages, limitations & challenges of defining psychotherapy

- **Advantage of a tight definition:** enables regulation, training standards and outcome research.
- **Limitation:** an overly narrow definition may exclude legitimate relational/humanistic work that resists manualisation (McWilliams). (McWilliams, p.8)

- **Challenge:** boundary overlap with counselling, coaching and psychiatric care makes any single definition contestable (Boulder critique). (Corsini, p.592)

Supplementary depth — "What is psychotherapy?" as an exam essay spine

A strong answer (i) opens with the definitional ambiguity (Wolberg; Boulder), (ii) gives Wolberg's working definition verbatim, (iii) unpacks its five core concepts, (iv) contrasts operational vs understanding-based traditions, and (v) closes by situating it against counselling, assessment and psychiatric treatment. This structure doubles as revision scaffolding for Topics 1–2.

1.9 Possible examination questions

1. Define psychotherapy. Why is it considered one of the most ambiguous terms in mental health? Trace its etymology and the "talking cure." (Wolberg; Notes-Psychotherapies)
2. Differentiate psychotherapy from counselling, psychological assessment and psychiatric treatment.
3. "Psychotherapy is an undefined technique..." — critically discuss the Boulder-era view of definition. (Corsini)
4. Discuss psychotherapy as a "unilateral professional relationship." What limits and duties follow from this? (Notes-Psychotherapies)
5. Compare how CBT, psychodynamic and humanistic traditions implicitly define psychotherapy by their "agent of change."

EXAM-ORIENTED SUMMARY

- **Etymology:** Greek *psyche* (soul) + *therapeutikos* (caring for another) "caring for another's soul"; Anna O.'s "talking cure" (Notes-Psychotherapies, p.0).
- **One-line definition:** treatment, by *psychological means*, of *emotional problems*, by a *trained person* in a *deliberate professional relationship*, aiming at symptom change + pattern change + growth (Wolberg, p.29).
- **APA/Norcross (1990):** "informed and intentional application of clinical methods and interpersonal stances derived from established psychological principles..."
- **Structural feature:** a *unilateral professional relationship* bounded in time, frequency, content and intimacy; confidentiality (with limits) + no dual relationships are built in.
- **Four common factors:** confidence in therapist · hope/ expectancy · conceptual rationale · sanctioned healing setting. ~70–80% improve (Smith et al., 1980).
- **Key tension:** operational/CBT definition vs anti-reductionist psychodynamic/humanistic view. **Boulder critique:** no universal agreement on definition, method or value. **Mnemonic:** TRIP-E.

1.10 References

Wolberg-Technique p.29, p.41 · Corsini p.592 · Wolberg-506Q p.16 · McWilliams p.8 · Notes-Psychotherapies

TOPIC A

2 · Objectives of Psychotherapy

Why we do it — the canonical goal statement, how goals are framed across traditions, and how global goals become measurable objectives.

2.1 Introduction

The ***objectives*** *(the intended ends or purposes of the treatment)* of psychotherapy follow directly from the definition. Clear objectives guide technique selection, allow progress to be monitored, and protect the client from aimless or interminable treatment. Objectives are negotiated *collaboratively*, not imposed. (Corey, p.63)

A crucial opening caution: "the list of possible goals of psychotherapy is **endless**" — behavioural change, better relationships, insight, support, or concrete outcomes such as keeping a job, a partner, staying out of hospital, or staying alive. Goals "may also be different for the *patient*, for the *significant others* … and for the *psychologist*." For example, the patient may want anxiety reduction, the psychologist may want increased insight, and a spouse may want increased independence. (Notes-Psychotherapies, p.3) Because of this, objectives must be *explicitly negotiated*; unspoken, divergent goals are a major source of drift, dropout and ethical difficulty (links to Topics 4 and 8).

Overall, psychotherapy "is intended to enhance the client's **mental health, flexibility, quality of relationships, and well-being**" through change in "cognition, perception, self-understanding, relational capacities, and overt behavior." For children and adolescents, collaborative work with families and schools promotes complementary change. (Notes-Psychotherapies, p.2)

The ethical grounding of all objectives — beneficence & nonmaleficence

WHY THE AIMS ARE WHAT THEY ARE

"If we apply the traditional medical ethical principles of ***beneficence*** *(acting for the patient's good)* and ***nonmaleficence*** *(doing no harm)*, then psychotherapy … should aim to help the patients with their problems, make them feel better and **do them no harm**." Most therapists pursue this by increasing the patient's **capacity for self-reflection** and awareness of the links between feelings and actions. (Notes-Psychotherapies, p.4)

2.2 The canonical objectives (source: Wolberg)

WOLBERG'S THREE OBJECTIVES

1. **Removing, modifying or retarding existing symptoms** — e.g. reducing panic attacks or low mood.
2. **Mediating (changing) disturbed patterns of behaviour** — altering maladaptive interpersonal or coping patterns.
3. **Promoting positive personality growth and development** — fostering maturation, resilience and fuller functioning.

(Wolberg-Technique, p.29)

2.3 Kleinke's six common therapeutic goals

Cutting across the 450+ schools, Kleinke (1994) identified **six fundamental therapeutic goals common to almost all psychotherapies** — the single most citable "objectives" list for an exam because it names the shared *change processes* rather than any

one school's technique: (Notes-Psychotherapies, p.3–4) *(Kleinke, 1994, cited in Notes-Psychotherapies.)*

1. **Overcoming demoralisation and gaining hope** — restoring the expectancy that change is possible (links to the "hope" common factor, Topic 1).
2. **Enhancing mastery and *self-efficacy*** *(the belief in one's capacity to succeed at a task)* — building a sense of competence and agency.
3. **Overcoming avoidance** — approaching, rather than escaping, feared situations and emotions.
4. **Becoming aware of one's misconceptions** — identifying and correcting distorted beliefs about self, others and the world.
5. **Accepting life's realities** — coming to terms with what cannot be changed.
6. **Achieving insight** — understanding the links between history, feelings and current patterns.

2.4 Core concepts & theoretical foundations

- **Adaptive change in affect, behaviour and mentation.** Corsini (Dumont) frames the "core objectives" as adaptive change across feeling, action and thought, helping the patient "fashion alternative and future memories" and build social, interpersonal, self-disciplinary and technical skills — described neurologically as learning-driven neuronal restructuring. (Corsini, p.23)
- **Collaborative goal-setting.** Corey: the therapist should discuss general goals (and length, duration, confidentiality and methods) early, developing goals *with* the client. (Corey, p.63)
- **Over-responsibility is a risk.** Assuming full responsibility for outcomes disempowers clients and courts ***burnout*** *(emotional exhaustion and reduced efficacy from chronic occupational stress)*. (Corey, p.63)
- **Goals can conflict with the modality.** "In CBT, the patient may want to remove all their negative emotion, while the therapist wants to **decrease excess** emotion"; CBT may relieve symptoms short-term while "psychodynamic therapies may not" — the

clinician's goal "may not be what the patient sees as the goal of therapy." Surfacing this difference is itself a therapeutic task.

(Notes-Psychotherapies, p.4)

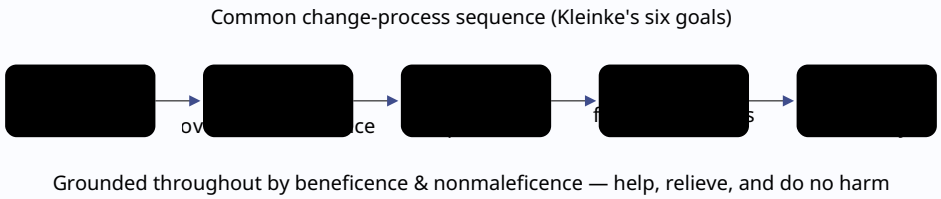


Figure 2.1 — From hope to growth: the common change process. *Kleinke's six shared goals read as a progression — restoring hope, building mastery and overcoming avoidance, correcting misconceptions and accepting realities, achieving insight, and promoting growth — all bounded by beneficence and nonmaleficence.* ***(Notes-Psychotherapies, p.3–4)***

2.5 From goals to measurable objectives (Jongsma)

Treatment planning distinguishes two levels, a key exam distinction:

Table 2.1 — Long-term goals vs short-term objectives (Jongsma)

Feature	Long-term goals	Short-term objectives
Scope	Broad, global	Narrow, specific
Measurability	*Need not* be measurable	Behavioural, measurable / quantifiable (e.g. attach a target number)
Function	State the desired positive outcome / direction	Track incremental, demonstrable progress

(Jongsma, p.24; p.25)

2.6 Clinical application & examples

ILLUSTRATIVE EXAMPLE

Illustrative teaching vignette. For a client with social anxiety, a **long-term goal** might be "reduce social anxiety and participate comfortably in work meetings." A matched **short-term objective** might be "attend 2 team meetings per week and speak at least once, rated for anxiety 0–10, over 4 weeks." The goal gives direction; the objective makes progress auditable (links to Topics 7 and 9).

2.7 Mnemonic

SMP-G OBJECTIVES

- **S** **Symptom** removal / modification / retardation.
- **M** **Maladaptive-pattern** mediation (behaviour change).
- **P** **Personality growth** and development.
- **G** **Goals set collaboratively & measurably** (Corey + Jongsma).

Apply: *Writing a treatment plan, SMP-G reminds you to cover symptom relief, pattern change and growth, then operationalise each as a measurable objective agreed with the client.*

2.8 Advantages, limitations & challenges

- **Advantage:** explicit objectives enable outcome measurement and third-party accountability.
- **Limitation / tension:** symptom-focused measurable objectives (CBT/behavioural) may under-represent insight and personality-growth aims valued by psychodynamic work.

- **Challenge:** balancing the client's priorities with clinically indicated goals without the therapist over-owning the outcome. (Corey, p.63)

2.9 Possible examination questions

1. Enumerate and explain Wolberg's three objectives of psychotherapy. (Wolberg)
2. Describe Kleinke's six common therapeutic goals shared across the schools of psychotherapy. (Notes-Psychotherapies)
3. "The list of possible goals of psychotherapy is endless." How are objectives grounded in beneficence and nonmaleficence, and why can patient and therapist goals conflict? (Notes-Psychotherapies)
4. Distinguish long-term goals from short-term objectives with examples. (Jongsma)
5. How are therapeutic goals set collaboratively, and why is over-responsibility a risk? (Corey)

EXAM-ORIENTED SUMMARY

- **Three objectives (Wolberg):** symptom change · pattern change · personality growth.
- **Kleinke's six common goals:** hope · mastery/self-efficacy · overcome avoidance · correct misconceptions · accept realities · insight (Notes-Psychotherapies p.3–4).
- **Ethical grounding:** beneficence + nonmaleficence — help, relieve, do no harm (p.4).
- **Goals are plural & party-dependent** (patient vs significant others vs psychologist) and can conflict with the modality — negotiate explicitly.
- **Corsini:** adaptive change in affect, behaviour, mentation. **Jongsma:** goals = broad (not necessarily measurable); objectives = behavioural, measurable. **Corey:** set goals collaboratively; avoid over-responsibility.
- **Mnemonic:** SMP-G.

2.10 References

Wolberg-Technique p.29 · Corsini p.23 · Corey p.63 · Jongsma p.24, p.25 · Notes-Psychotherapies p.2, p.3, p.4

TOPIC B

3 · Training Professional Therapists

How therapists are made — the self as instrument, structure of training, supervision, personal therapy, continuing professional development and competence measurement.

3.1 Introduction

Because the definition of psychotherapy builds in "a trained person," the quality of ***training*** *(the formal, supervised preparation of therapists)* is central to the field's legitimacy and to client safety. Training is unusual among professions because the practitioner's own personality is a primary working tool.

Training is also, fundamentally, a **risk-management system**. The Notes put it bluntly: "Negligence and incompetence are potential risks for all clinicians, and these can (and should) be addressed by **training, licensing or registration, and supervision processes**." (Notes-Psychotherapies, p.5) On this view the whole pipeline — selection, training, personal therapy, supervision and continuing development — exists to protect clients from a practitioner's blind spots and to make competence demonstrable rather than assumed. In India this is operationalised by the ***Rehabilitation Council of India (RCI)*** *(the statutory body that regulates training and maintains the register of clinical psychologists)* under the RCI Act, 1992 — stated generally; verify against the primary statute.

3.2 Core concept — the self as instrument

LISTER-FORD

"The chief instrument for most psychotherapists is the self." Effective practice is a "well-blended synthesis of art, science and reflection," and trainees must "come to know themselves at least as well as the theories they are studying," usually through ***personal therapy*** *(the trainee undergoing their own therapy to increase self-awareness and manage personal reactions)*.

(Lister-Ford, p.120)

3.3 Structure and length of training

- **UK / Europe (Lister-Ford).** The UKCP (UK Council for Psychotherapy) requires trainees to be 25 before commencing; minimum training length is 3–4 years depending on the regulatory body; the European minimum (EAP, European Association for Psychotherapy) is 4 years; most qualify around age 30. Required personal qualities: maturity, sensitivity, and the capacity for self-reflection — using one's own responses as a "barometer."

  (Lister-Ford, p.121)

CONTEXT NOTE

These age/length figures are UK/European regulatory requirements reported by Lister-Ford (2007); Indian training routes (e.g. the RCI-regulated M.Phil. Clinical Psychology) differ. See Topic 4 for the Indian regulatory frame, stated at a general level.

3.4 Supervision, personal therapy & CPD

- **Supervision & monitoring.** The CORE (Clinical Outcomes in Routine Evaluation) system monitors effectiveness and helps identify ***continuing professional development*** (CPD) *(ongoing,*

lifelong learning to maintain and extend competence after qualification) needs. (Lister-Ford, p.118)

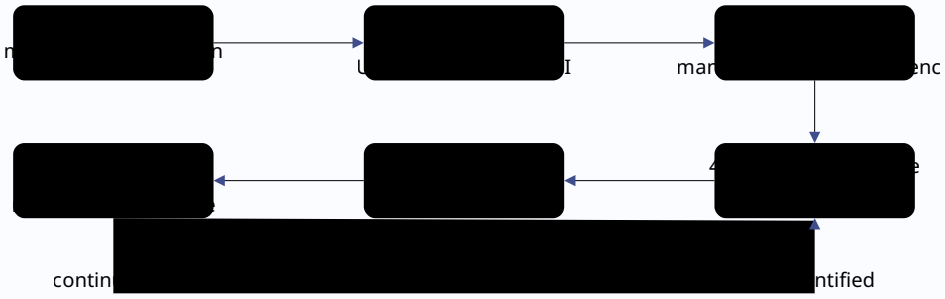
- **Personal therapy & countertransference.** Corey: personal therapy helps the counsellor recognise and manage ***countertransference*** *(the therapist's own, often unconscious, emotional reactions to the client that can distort the work)*; beginning-therapist issues include anxiety, perfectionism and tolerating ambiguity. (Corey, p.63; p.12)
- **Oral tradition & workforce risk.** McWilliams warns new therapists may carry caseloads of ~60 patients "with no provision for supervision or continuing education," and that much learning is an "oral tradition" transmitted by supervisors and personal therapy. (McWilliams, p.8; p.9)
- **Manualised training.** The MET manual notes therapists require "intensive training and supervision" on a standardised protocol; Project MATCH documented procedures to select, train and supervise therapists. (MET, p.13)

3.5 Measuring competence & fidelity

- **CTRS (Beck).** The ***Cognitive Therapy Rating Scale*** (CTRS) is "used in many research studies and training programs to measure therapist competency," alongside a Cognitive Case Write-Up based on the Academy of Cognitive Therapy certification template. (Beck-CBT, p.16)
- **Fidelity vs competence.** ***Fidelity*** *(adherence — delivering the protocol as intended)* and ***competence*** *(skilful delivery)* are distinct; EBP dissemination depends on both (see Topic 10). The 1995 Division 12 report was titled *Training in and Dissemination of Empirically-Validated Psychological Treatments*. (Hofmann-ArtScience, p.53)
- **Assessment skill is a core, trainable competence.** "Successful psychotherapy matches the right therapy and right therapist to the patient, which is why therapists need to develop **good quality assessment skills, not just for their own discipline but for others**" — being able to formulate the same problem in several

ways. Therapist characteristics (competence *and* personality features) themselves influence process and outcome.
(Notes-Psychotherapies, p.2; p.4–5)

- **Record-keeping supports thinking and supervision.** Keeping a record "is a way to do this; and also helps with the process of **thinking and supervision**" — the clinical record is not only a legal document but a reflective tool that feeds supervision (links to Topic 7). (Notes-Psychotherapies, p.6)



nale: training + licensing/registration + supervision mitigate negligence & incompetence (Notes-Psych

Figure 3.1 — The therapist-development pipeline. *Selection → formal training → personal therapy → supervised practice → competence/fidelity measurement (e.g. CTRS) → lifelong CPD, with a feedback loop that re-measures competence. The whole system exists to manage the risks of negligence and incompetence.* ***(Lister-Ford p.118–121; Beck-CBT p.16; Notes-Psychotherapies p.5)***

3.6 Comparison table — how traditions train & assess competence

Table 3.1 — Training emphases across traditions

Element	CBT / behavioural	Psychodynamic	Shared pillars
Competence measure	Rating scales & certification (CTRS) (Beck-CBT, p.16)	Supervisory judgement; "oral tradition" (McWilliams, p.9)	Supervised practice
Self-work	Reflective practice, self-monitoring	Personal analysis / therapy central (Lister-Ford, p.120)	Personal therapy for self-awareness
Protocol use	Manuals & fidelity checks (MET, p.13)	Frame & process, less manualised	Ethics & CPD throughout (Lister-Ford, p.118)

3.7 Clinical application

ILLUSTRATIVE EXAMPLE

Illustrative teaching vignette. A first-year trainee notices intense irritation towards a demanding client. Rather than acting on it, she brings it to supervision and personal therapy, recognising it as countertransference. She also audio-records a session (with consent) to be rated on the CTRS, converting self-awareness into measurable competence feedback.

3.8 Mnemonic

SUPER-V TRAINING

- **S** **Self-awareness** — the self is the chief instrument.
- **U** **Understanding of theory** — know theories as well as yourself.
- **P** **Personal therapy** — manage countertransference.
- **E** **Evaluation of competence** — fidelity/competence measures (CTRS).
- **R** **Reflective practice & CPD** — lifelong learning (CORE).
- **V** **(Super)Vision** — ongoing supervision anchors it all.

Apply: *Audit your own development with SUPER-V — e.g. a trainee lacking supervision provision (the "V") is a safety risk McWilliams warns about.*

3.9 Advantages, limitations & challenges

- **Advantage:** supervised, self-aware, CPD-driven training protects clients and sustains competence.
- **Limitation:** competence measures (CTRS) fit manualised CBT better than open-ended psychodynamic work.
- **Challenge:** under-resourced services may provide little supervision/CPD, risking harm and burnout. (McWilliams, p.8)

3.10 Possible examination questions

1. "The chief instrument for most psychotherapists is the self." Discuss the implications for training. (Lister-Ford)
2. Describe the role of personal therapy and supervision in managing countertransference. (Corey, McWilliams)
3. How is therapist competence/fidelity measured? Discuss the CTRS. (Beck)

4. With a flowchart, describe the therapist-development pipeline and explain how it manages the risks of negligence and incompetence.
(Notes-Psychotherapies; Lister-Ford)

EXAM-ORIENTED SUMMARY

- **Self as instrument** personal therapy + self-knowledge (Lister-Ford p.120).
- **Pillars:** training length/standards · supervision · personal therapy · CPD (CORE).
- **Competence:** CTRS measures CBT therapist competency (Beck p.16); fidelity ≠ competence.
- **Pipeline as risk-management:** training + licensing/ registration + supervision mitigate negligence & incompetence (Notes-Psychotherapies p.5); assessment skill is a core trainable competence (p.4–5).
- **Risk:** no supervision/CPD + huge caseloads (McWilliams).
- **Mnemonic:** SUPER-V.

3.11 References

Lister-Ford p.118, p.120, p.121 · Corey p.12, p.63 · McWilliams p.8, p.9 · MET p.13 · Beck-CBT p.16 · Hofmann-A

TOPIC C

4 · Ethical & Legal Issues

The ethical spine of practice — informed consent, confidentiality vs privilege, exceptions/duty to report, boundaries and dual relationships, technology ethics, a decision-making framework, and the Indian legal frame (general level).

4.1 Introduction

Ethics *(professional standards of right conduct that protect clients and the profession)* and ***law*** *(binding rules enforced by the state)* overlap but are not identical. A key exam skill is distinguishing *ethical guidance* (professional codes, aspirational) from *binding law* (statutes, enforceable). Where they conflict, practitioners must reason carefully, document, and often consult.

The Notes offer an elegant definition of ethics itself: any discussion of an ethical dilemma "involves a special type of dialog; the **discourse of ought and should** in interpersonal relationships" — how therapists *ought* to behave and what they *should* do in difficult situations with patients and colleagues. There is a "close kinship" between ethics and therapy: "just as ethical debate is all about how individuals should treat other people … so psychotherapy explores interpersonal relating." (Notes-Psychotherapies, p.8) Crucially, **law is not automatically ethical**: legal cases "should not be understood as legal advice … nor does the law always provide an ethically justifiable source of guidance, as is clear if we remember the impact of both the Nazi and South African race laws." (Notes-Psychotherapies, p.8–9)

Why this matters so much: there are "**over 450 schools of psychotherapy** currently in existence (Simon, 2001, p. 90)," yet "basic clinical–legal principles" apply across most of them, and negligence claims against psychotherapists have "substantially increased." Every clinician needs a working knowledge of these principles to avoid "maladaptive defensive practices." (Notes-Psychotherapies, p.7) *(Simon, 2001, cited in Notes-Psychotherapies.)* Ethical dilemmas cluster in three

domains: "(1) **goals and objectives of therapy**; (2) **the boundaries between their different identities**; (3) the **social and political frameworks** in which they work." The overarching duty is simply "to **make the best quality decision that can be made**" (one of fourteen approaches to ethical reasoning catalogued by Tjelveit, 1999). (Notes-Psychotherapies, p.8–9) *(Tjelveit, 1999, cited in Notes-Psychotherapies.)*

THE APA ETHICS CODE — THE STANDARDS MOST OFTEN EXAMINED

The ***APA Ethical Principles of Psychologists and Code of Conduct*** *(the APA's binding-on-members ethics code, 2002, amended 2010 & 2017)* opens with five aspirational **General Principles** — *A: Beneficence & Nonmaleficence; B: Fidelity & Responsibility; C: Integrity; D: Justice; E: Respect for People's Rights & Dignity* — and then enforceable **Ethical Standards**, including **Informed Consent** (Standard 3.10 general; 10.01 therapy-specific), **Confidentiality** (Standard 4, with its limits and mandated disclosures), and **Multiple Relationships** (Standard 3.05). (APA Ethics Code, apa.org/ethics) *Stated at a general level; verify exact standard wording and numbers against the live APA Ethics Code. These are professional-guideline standards, distinct from binding statute.*

4.2 Informed consent (source: Corey)

COREY

"Informed consent is an ethical and legal requirement that is an integral part of the therapeutic process." It establishes the foundation for the working alliance and is "far broader than simply making sure clients sign the appropriate forms." (Corey, p.75)

Informed consent *(the client's voluntary, knowing agreement to treatment after disclosure of relevant information)* elements include:

general goals, counsellor and client responsibilities, the **limitations of and exceptions to confidentiality**, legal/ethical parameters, and the practitioner's qualifications. Consent may be written, oral or combined; if oral, the therapist **must document** its nature and extent in the clinical record. (Corey, p.75; p.76)

CONSENT IS A LEGAL DUTY — AND PSYCHOTHERAPY HAS RISKS

Consent is not a courtesy. "Some states that have informed consent statutes declare the **failure to obtain informed consent to be negligence** (Slovenko, 1989)." Even a purely "talking" treatment needs informed consent because "**all psychotherapies have risks and benefits**" — for example untoward transference reactions, regressive dependency, or a general worsening before improvement. (Notes-Psychotherapies, p.7–8) *(Slovenko, 1989, cited in Notes-Psychotherapies.)* This is also why the therapist acquires a ***legal duty of care*** *(an enforceable obligation to practise with reasonable skill)*: to practise "in accordance with a reasonable body of … psychotherapeutic opinion," make logical treatment decisions, conform to ethical guidance, obtain informed consent, tell the truth when asked, and abide by mandatory-reporting law. (Notes-Psychotherapies, p.10)

4.3 Confidentiality vs privileged communication

Table 4.1 — Confidentiality vs privileged communication (Corey)

	Confidentiality	Privileged communication
Nature	*Ethical* concept (and in most jurisdictions a legal duty)	*Legal* concept
Protects	Client information from disclosure generally	Client from having confidential communications revealed *in court*
Key limit	Has exceptions (see 4.4)	Generally does **not** apply to group, couples, family, or child/adolescent therapy (>2 people in the room)

(Corey, p.76)

4.4 Exceptions to confidentiality / duty to report

MANDATORY BREACHES (COREY)

Confidentiality must be broken, as a legal requirement, for: **child abuse, elder abuse, abuse of dependent adults, and danger to self or others.** (Corey, p.77)

Four sources of the duty of confidentiality

The duty is not a single rule but a convergence of four sources, worth listing in an answer: (Notes-Psychotherapies, p.7)

1. **Ethical codes** of the mental-health professions (e.g. APA Standard 4).
2. **Professional licensure regulations** and confidentiality & privilege *statutes*.
3. **Common-law privilege** — the attorney–client privilege extended (less stringently) to psychotherapists by case law.
4. **Constitutional privacy** guarantees (noting that no *explicit* constitutional right of privacy exists in that source).

A landmark illustration is ***Jaffe v. Redmond (1996)*** *(the US Supreme Court case)*, which held that psychotherapist–patient communications

are confidential/privileged in **federal** trials (though it did not bind state courts). (Notes-Psychotherapies, p.7) *(US case law reported in Notes-Psychotherapies; illustrative of the principle, not Indian law.)*

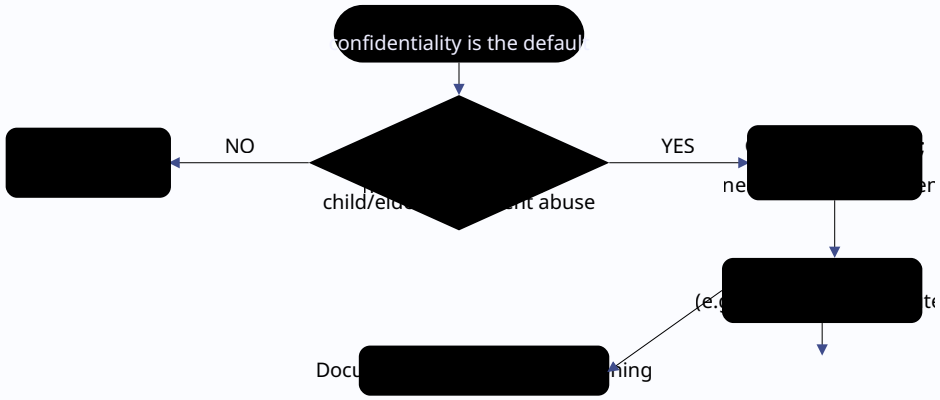


Figure 4.1 — Confidentiality-limits / duty-to-report decision flow.
Confidentiality is the default; a mandatory exception (danger to self/others, child/elder/dependent-adult abuse) switches the clinician into consult disclose-minimum report-per-law document.
***(Corey p.77; Notes-Psychotherapies p.7, p.10)** Indian reporting duties (e.g. POCSO) are general — verify against the primary statute.*

4.5 Boundaries & dual/multiple relationships

- **Sexual involvement — absolute prohibition.** Corsini (Pope & Wedding): "No circumstances or rationale justify sexual involvement with a patient"; the prohibition predates the Hippocratic Oath. Harm clusters into ten areas (e.g. guilt, emptiness/isolation, impaired trust, confused roles/boundaries, emotional lability, suppressed rage, increased suicidal risk, cognitive dysfunction). A survey of 5,148 therapists found ~4.4% reported sex with a patient (male 6.8% vs female 1.6%).
  (Corsini, p.597; p.598)
- **Nonsexual touch & multiple relationships.** ***Dual / multiple relationship*** *(having a second, non-therapeutic role with a client, e.g. business or social, that risks impaired judgement or exploitation)*. Nonsexual touch is distinct from sexual involvement and has no inherent harm, but context, culture, competence and consent must be weighed; the same gender patterns extend to supervisor–supervisee and professor–student relationships. (Corsini, p.597; p.598)

4.6 Technology / telepsychology ethics

Corey notes that Section H of the ACA (American Counseling Association) Code of Ethics (2014) governs ***technology*** and ***telepsychology*** *(delivering psychological services remotely via phone/ video/internet)* — covering computer-mediated communication, social media as a delivery platform, distance-counselling informed consent and security, client verification, records, and professional boundaries.

(Corey, p.76; p.77)

4.7 An ethical decision-making framework (with mnemonic)

When codes do not give a clear answer, trainees apply a structured ***ethical decision-making*** process. The flowchart and mnemonic below map a defensible, documentable sequence; it operationalises Corey's emphasis that clients should be *included* in ethical decisions and that reasoning be documented, and Tjelveit's overarching duty "to make the best quality decision that can be made."

(Corey, p.75; p.76; Notes-Psychotherapies, p.8)

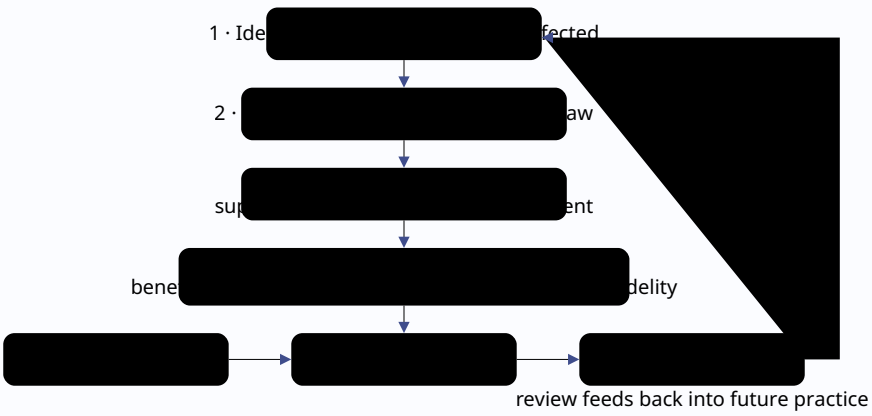


Figure 4.2 — Ethical decision-making flowchart. *Identify the dilemma identify the relevant code and law (kept separate) consult/consider options and include the client weigh the five principles act document review. Mirrors the IDENTIFY mnemonic below.*

(Corey p.75–76; Notes-Psychotherapies p.8; APA General Principles A–E)

IDENTIFY ETHICAL FRAMEWORK

- **I** **Identify** the problem and who is affected.
- **D** **Determine** the relevant ethical codes *and* laws (separate the two).
- **E** **Examine** client rights, autonomy and context (culture, capacity, minors).
- **N** **Name** the possible courses of action.
- **T** **Think through** consequences of each option for all parties.
- **I** **Involve** the client and consult a supervisor/colleague.
- **F** **Finalise** and act on the best-reasoned option.
- **Y** **Yield a record** — document the reasoning and review the outcome.

Apply: *A client hints at harming an ex-partner. IDENTIFY you separate the legal duty-to-warn/report from ethical confidentiality (D), involve a supervisor (I), act, and document (Y) — matching Corey's duty-to-report exceptions (p. 77).*

4.8 Indian legal & professional frame (general level)

INDIAN STATUTES — GENERAL, NON-CLAUSE-SPECIFIC OVERVIEW

The following are **statutes/guidelines relevant to Indian practice**. They are stated at a general, accurate level only; they are *binding law*, distinct from professional ethical guidance, and are **not** among the repository source texts. **Always verify against the primary statute** — specific section/clause numbers are deliberately not quoted here.

- **Rehabilitation Council of India (RCI) Act, 1992** — governs training standards and the register of rehabilitation professionals, including clinical psychologists in India (regulatory/statutory). *Ethical guidance vs binding law: the Act is binding; professional codes layer on top.*
- **Mental Healthcare Act (MHCA), 2017** — rights-based mental-health law addressing, at a general level, the rights of persons with mental illness, informed consent, confidentiality of records, and advance directives.
- **Protection of Children from Sexual Offences (POCSO) Act, 2012** — child-protection statute; relevant to mandatory reporting duties when working with minors.
- **Digital Personal Data Protection (DPDP) Act, 2023** — data-protection law relevant to the storage and processing of clients' personal/health data (records, telepsychology).

Caution: the above are summarised from general professional knowledge, not the repository PDFs. Scope, definitions and obligations must be confirmed in the current official text of each statute before relying on them in practice or in an answer.

4.9 Clinical application

ILLUSTRATIVE EXAMPLE

Illustrative teaching vignette. During intake, a therapist explains (and documents) the limits of confidentiality. Three sessions in, an adolescent client discloses ongoing abuse. The therapist recognises this triggers a mandatory-report exception (danger/ child abuse, Corey p.77; in India, child-protection obligations — verify against the primary statute), consults a supervisor, informs the client within the consent framework already set, and records the decision.

4.10 Advantages, limitations & challenges

- **Advantage:** clear ethics + informed consent build trust and protect both parties.
- **Limitation:** codes vary by jurisdiction; privilege rules differ across settings.
- **Challenge:** genuine conflicts between confidentiality and duty to protect; evolving telepsychology and data-protection risks.

(Corey, p.76–77; Corsini, p.597–598)

4.11 Possible examination questions

1. Discuss informed consent as an ethical and legal requirement. What must it include? (Corey)
2. Differentiate confidentiality from privileged communication and list the exceptions to confidentiality. (Corey)
3. Why is sexual involvement with a client absolutely prohibited? Discuss boundary and dual-relationship issues. (Corsini)
4. Outline an ethical decision-making framework and apply it to a duty-to-report scenario.

5. "Ethics is the discourse of ought and should." Discuss the relationship between ethics and law in psychotherapy, noting that law is not automatically ethical. (Notes-Psychotherapies)
6. Summarise the APA Ethics Code's General Principles (A–E) and the standards on informed consent, confidentiality and multiple relationships. (APA Ethics Code)
7. What are the four sources of the duty of confidentiality? Illustrate with *Jaffe v. Redmond*. (Notes-Psychotherapies)

EXAM-ORIENTED SUMMARY

- **Ethics** = "the discourse of ought and should"; **law is not automatically ethical** (Notes-Psychotherapies p.8–9). Dilemmas cluster in goals · identity-boundaries · socio-political frame.
- **APA Ethics Code:** General Principles A–E (A = Beneficence/ Nonmaleficence); Standards 3.10/10.01 consent · 4 confidentiality · 3.05 multiple relationships (verify live).
- **Informed consent** = ethical + legal; broader than a signed form (Corey p.75); failure to obtain it can be negligence (Slovenko 1989); all therapies have risks (p.7–8).
- **Confidentiality** (ethical/legal duty, four sources) vs **privilege** (legal, court; not in group/family/child therapy) — Corey p.76; *Jaffe v. Redmond* 1996.
- **Mandatory breaches:** child/elder/dependent-adult abuse; danger to self/others (Corey p.77).
- **Boundaries:** absolute ban on sexual involvement; weigh dual relationships (Corsini p.597–598).
- **Indian law (general):** RCI 1992 · MHCA 2017 · POCSO 2012 · DPDP 2023 — binding law, verify the primary statute.
- **Mnemonic:** IDENTIFY.

4.12 References

Corey p.75, p.76, p.77 · Corsini p.592, p.597, p.598 · Notes-Psychotherapies p.7, p.8, p.9, p.10 (Simon 2001, Slo

TOPIC D

5 · Rights & Responsibilities

The collaborative contract — client rights, shared responsibilities, the therapist's duties, and how these map onto informed consent.

5.1 Introduction

Rights and responsibilities make the therapeutic relationship a ***collaborative partnership*** *(a working alliance in which duties and decisions are explicitly shared rather than imposed)*. Corey frames informed consent itself as the vehicle for client rights: it "involves the right of clients to be informed about their therapy and to make autonomous decisions pertaining to it," turning clients into "active partners and true collaborators." (Corey, p.75)

Rights and responsibilities are two sides of one coin: **every client right implies a matching therapist duty**. The Notes state the core pairing directly: "Patients have the **right** and psychotherapists the **duty** to have spoken or written communications during the course of treatment kept confidential." (Notes-Psychotherapies, p.7) The underpinning ethical principle is ***respect for autonomy*** *(the client's freedom to choose or refuse treatment for themselves)*: "patients should be free to choose or refuse for themselves what treatment they have." (Notes-Psychotherapies, p.4)

5.2 Client rights (sources: Corey; Notes-Psychotherapies)

- **Right to be informed & to decide (autonomy).** Clients have the right to information about therapy and to make autonomous decisions about it — including the right to *refuse* or withdraw; educating clients about rights and responsibilities empowers them and builds trust. (Corey, p.75; Notes-Psychotherapies, p.4)
- **Right to confidentiality (within limits).** The client holds the right, and the therapist the duty, that communications be kept

confidential — subject to the documented exceptions in Topic 4. (Notes-Psychotherapies, p.7)

- **Right to be warned of the risks of therapy.** The Notes argue that "few therapists warn patients of the risks of a proposed psychotherapy, although the potential benefits may be emphasized" — yet clients *should* be told "that the process of psychotherapy is not always comfortable, and that **many patients feel worse before they feel better**." This is a genuine, often-neglected right flowing from informed consent. (Notes-Psychotherapies, p.7; p.4)
- **Right to know about supervision.** Clients have a right to know their therapist may discuss details of the relationship with a supervisor or colleague. (Corey, p.76)
- **Rights embedded in consent elements.** The "client bill of rights" maps directly onto the informed-consent elements — goals, responsibilities, confidentiality limits, parameters and qualifications (Topic 4). (Corey, p.75)

5.3 Shared & therapist responsibilities

- **Two responsibility errors (Corey).** (a) Assuming *full* responsibility for direction/outcome — disempowers clients and risks burnout; (b) *refusing* responsibility for accurate assessment and treatment planning. How responsibility is shared "should be addressed early in the course of counseling." (Corey, p.63)
- **Therapist's explicit duties.** Discuss length and duration of sessions, confidentiality, general goals, and methods. (Corey, p.63)
- **Responsibility for the whole person (Corsini/Dumont).** Therapists are "responsible to some degree for both the natural and nurturant components of the patients' lives"; prescribing psychoactive medication "without assuming responsibility for the patient's concurrent experience" is described as irresponsible. (Corsini, p.23; p.24)
- **Duty to third parties — and to the client's confidentiality.** When family members or employers contact the therapist, the

therapist "may have to **balance the patient's claim to honesty and confidentiality against a possible harm** to them," and must *seek the patient's consent before disclosing*. The Notes reframe confidentiality itself as "the principle of **informed consent to disclosure.**" (Notes-Psychotherapies, p.5; p.6)

- **Duty not to exploit the relationship.** "A psychologist will not be effective if he or she tries to be a friend, a colleague, or a lover"; the professional relationship must not be compromised by dual relationships — the therapist's core fiduciary responsibility. (Notes-Psychotherapies, p.1)



Figure 5.1 — Client rights therapist responsibilities (the collaborative partnership). *Each right on the left has a matching duty on the right; informed consent is the contract that pairs them. "Patients have the right and psychotherapists the duty…" (Corey p.63–76; Notes-Psychotherapies p.1, p.4–7)*

5.4 Comparison table — rights vs responsibilities

Table 5.1 — Client rights mapped against responsibilities (both parties)

Domain	Client rights	Client responsibilities	Therapist responsibilities
Information & consent	To be informed; to give/withhold consent; autonomy (Corey, p.75)	To engage honestly; to ask questions; to decide	To disclose goals, methods, qualifications, confidentiality limits (Corey, p.75)
Confidentiality	To privacy within legal limits	To understand the exceptions	To protect records & explain limits/ exceptions (Corey, p.76–77)
Direction of work	To participate in goal-setting	To share responsibility for progress (homework, attendance)	Accurate assessment & planning; avoid over/under-responsibility (Corey, p.63)
Oversight	To know about supervision (Corey, p.76)	To raise concerns	To seek supervision; practise within competence

5.5 Mnemonic

RIGHTS RIGHTS & DUTIES

- **R** **Right to information** about therapy.
- **I** **Informed, autonomous decisions** (consent/refusal).
- **G** **Goals set together** (collaborative direction).
- **H** **Honesty & shared responsibility** (client's part).
- **T** **Transparency about supervision** & limits.
- **S** **Safety & competence** — therapist's duty to assess, plan & protect.

Apply: *At intake, run RIGHTS to script your opening contract — e.g. "T" reminds you to tell the client you discuss cases in supervision (Corey p.76).*

5.6 Clinical application

ILLUSTRATIVE EXAMPLE

Illustrative teaching vignette. A client expects the therapist to "fix" them passively. Using the shared-responsibility principle, the therapist clarifies early that progress depends on the client's active participation (e.g. between-session tasks), while the therapist remains responsible for competent assessment and planning — avoiding both over-responsibility (burnout) and under-responsibility (poor planning). (Corey, p.63)

5.7 Advantages, limitations & challenges

- **Advantage:** explicit, early negotiation of rights/responsibilities empowers clients and prevents disputes.
- **Limitation:** capacity issues (minors, serious illness) complicate "autonomous decisions" (see Topic 6).

- **Challenge:** balancing client autonomy with therapist duty to protect and to maintain standards.

5.8 Possible examination questions

1. Discuss client rights and therapist responsibilities as expressed through informed consent. (Corey)
2. "Patients have the right and psychotherapists the duty..." — explain how each client right implies a matching therapist responsibility. (Notes-Psychotherapies)
3. Discuss the client's right to be warned of the risks of therapy and the principle of respect for autonomy. (Notes-Psychotherapies)
4. What are the two errors therapists make regarding responsibility, and why do they matter? (Corey)
5. Construct a rights-vs-responsibilities diagram/table for the therapeutic relationship.

EXAM-ORIENTED SUMMARY

- **Core pairing:** patient's *right* + therapist's *duty* of confidentiality; underpinned by respect for autonomy (Notes-Psychotherapies p.4, p.7).
- **Rights** flow from informed consent: information + autonomous decisions (incl. refusal) + being warned of risks + knowing about supervision (Corey p.75–76; Notes p.7).
- **Shared responsibility**: avoid over-responsibility (burnout) and under-responsibility (poor planning); settle it early (Corey p.63).
- **Therapist duties**: disclose goals/methods/qualifications/ confidentiality limits; balance third-party requests (consent-to-disclosure); not exploit the relationship; assess, plan, supervise.
- **Mnemonic:** RIGHTS.

5.9 References

Corey p.63, p.75, p.76, p.77 · Corsini p.23, p.24 · Notes-Psychotherapies p.1, p.4, p.5, p.6, p.7. (Full source map

TOPIC E

6 · Issues of Consent & Assent in Minors

Who may agree to a child's therapy — the distinct roles of *consent* and *assent*, developmental capacity, confidentiality limits, safeguarding, and why there is no single universal rule.

6.1 Introduction

When the client is a ***minor*** *(a person below the legal age of majority; in India, below 18 years under the Indian Majority Act 1875 — general knowledge, verify against the primary statute)*, the ordinary adult model of autonomous informed consent cannot simply be transferred. A child may lack the legal standing and sometimes the developmental capacity to authorise their own treatment, yet forcing therapy on an unwilling child is both unethical and clinically self-defeating. The solution adopted across professional ethics is a *two-part* arrangement: legally valid **consent** from a parent or guardian, plus developmentally appropriate **assent** from the minor. (Corey, p.75; p.76)

NO SINGLE UNIVERSAL RULE

There is **no one rule** that governs every minor in every setting. What is required depends on (a) the *applicable law and jurisdiction*, (b) the *circumstances* (age, maturity, risk, whether abuse or self-harm is involved, custody arrangements), and (c) the *type of service* (routine counselling vs. emergency/crisis vs. research vs. court-ordered assessment). Trainees must check local statute and institutional policy rather than apply a blanket formula.

6.2 Definitions (identify source / author)

CONSENT VS. ASSENT

- ***Informed consent*** *(the legally and ethically valid authorisation to proceed, given by a person with the legal capacity to do so)* — for a minor this is normally given by a **parent or legal guardian**. Corey stresses informed consent is "an ethical and legal requirement … far broader than simply making sure clients sign the appropriate forms." (Corey, p.75)
- ***Assent*** *(the minor's own developmentally appropriate agreement to participate — an affirmative "yes" that is not, by itself, legally sufficient but is ethically essential)*. Assent respects the child as a person and is sought using language the child can understand. *(Assent as a term is general professional/ethics knowledge; the repository texts did not yield a page-level definition, so it is presented here as standard practice rather than cited to a PDF page.)*
- ***Guardian*** *(the adult with legal authority and responsibility for the minor — a parent, or a court-appointed legal guardian)*.

6.3 Core concepts — the three roles

Table 6.1 — Consent vs. assent: distinct roles in work with minors

Dimension	Consent (parent / guardian)	Assent (the minor)
Nature	Legally binding authorisation	Ethical agreement; not legally sufficient alone
Who gives it	Parent or legal guardian (has legal capacity)	The child/adolescent, per developmental capacity
Basis	Legal authority over the minor	Respect for the emerging autonomy of the child
Language	Full adult explanation of goals, methods, limits	Age-appropriate, simplified explanation
If refused	Therapy generally cannot lawfully proceed (except statutory/emergency exceptions)	Refusal is a clinical and ethical signal to address; may still proceed with consent but engagement suffers

- **Developmental capacity, maturity & comprehension.** The weight given to a minor's own views rises with age and maturity. A 6-year-old can give simple assent ("Is it okay if we play and talk about worries?"); a mature 16-year-old may have substantial comprehension approaching adult understanding. Capacity is assessed, not assumed from age alone.
- **Age-appropriate explanation.** For *children*, use concrete, play-based, concrete language and check understanding. For *adolescents*, explain goals, confidentiality and its limits more fully, acknowledging their need for privacy and autonomy.

6.4 Theoretical / professional foundations — autonomy & the capacity to consent

The ethical principle that underpins *all* consent — adult or minor — is **respect for autonomy**: patients should be free to choose or refuse for themselves what treatment they have. The new source states the legal test crisply: "for consent to be valid it must be given voluntarily,

by a patient who is competent to make that decision, and the patient must be adequately informed about what the treatment involves, including any possible negative side-effects." (Notes-Psychotherapies, p.4) This yields the three legal ingredients of valid consent — **voluntariness**, **capacity/competence**, and **adequate information** — each of which becomes problematic with minors.

CAPACITY / COMPETENCE (DEFINE AT FIRST USE)

Capacity (competence) *(the ability to understand relevant information, weigh it in the light of one's values, and communicate a choice — i.e. to exercise autonomy)*. The source notes capacity is "given by competent patients, who are capable of exercising their autonomy in a way that expresses their values and interests, while giving weight to conflicting values and facts." (Notes-Psychotherapies, p.5) Capacity is **decision-specific and can fluctuate**; it is assessed, not assumed from age or diagnosis.

- **Groups who may lack competence.** "In a general sense, most psychotherapy patients will be competent to make decisions about their own treatment … this may not be true for all." The source names **children**, **patients with psychotic disorders**, and **patients with learning disabilities**, and adds the recently traumatised or bereaved. Crucially, "the problem is usually not a global lack of competence, but rather that patients experience **fluctuating levels of capacity**, or experience rapid changes in how they make decisions." (Notes-Psychotherapies, p.5)
- **The child-refusal puzzle.** "It is hard to know what to make of a child's refusal to have therapy, which they agreed to only a day before. Can this be understood as an informed choice to refuse treatment? Or is this refusal merely evidence that the work has begun?" (Notes-Psychotherapies, p.5) A minor's day-to-day change of mind is therefore read clinically, not just as a binary consent/withdrawal.
- **Coercion & "true" voluntariness.** Some patients are *coerced* into therapy — "children who only agree under pressure from family,"

or forensic clients "mandated to have treatment, as an alternative to prison." Such pressures "raise dilemmas about the nature of 'true' voluntariness." The first ingredient of valid consent — voluntariness — is thus especially fragile for minors embedded in a family's "emotional network." (Notes-Psychotherapies, p.5)

- **Privilege does not extend to child/adolescent therapy.** Corey: ***privileged communication*** *(a legal protection preventing confidential communications from being revealed in court)* "does not apply to … child and adolescent therapy, or whenever there are more than two people in the room." The parent/guardian is typically involved, so the minor's communications are not shielded in the same way as an adult's. (Corey, p.76)
- **Collateral involvement in assessment.** Judith Beck lists *"Involving a Family Member"* as a formal part of the evaluation session — directly relevant when the client is a minor and collateral information, consent or assent is needed. (Beck-CBT, p.18 [ToC])

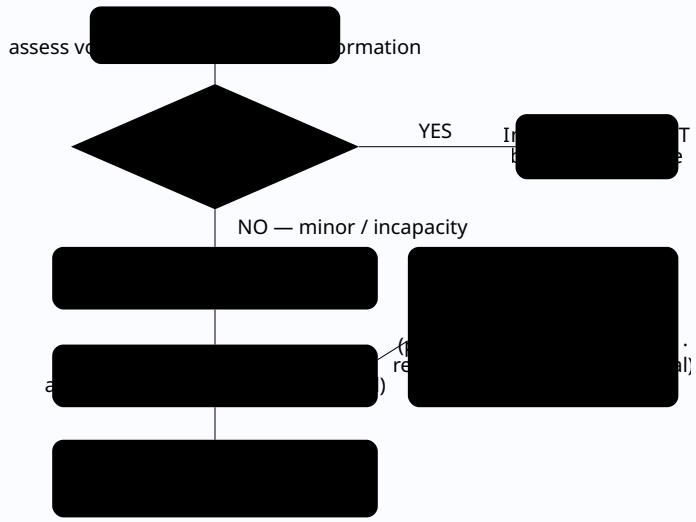


Figure 6.1 — Consent / assent decision flow. *An adult with capacity gives informed consent alone. A minor or an adult lacking capacity needs the **guardian's legally valid consent PLUS the minor's developmentally appropriate assent**. Capacity is assessed and re-assessed because it is decision-specific and may fluctuate (psychosis, learning disability, recent trauma, a child's day-to-day refusal). Any disclosure of abuse or danger triggers the safety override.*
(Notes-Psychotherapies, p.4–5; Corey, p.75–77)

6.5 Confidentiality, parental involvement & safeguarding

With minors, confidentiality is *shared* rather than absolute. Good practice is to agree at the outset, with both the adolescent and the guardian, **what will and will not be shared** — for example, that general progress is shared with parents but session details are kept private *unless* there is a risk of serious harm. The new source reframes confidentiality itself as "better understood as the principle of **informed consent to disclosure**": the therapist is "under an ethical duty to obtain their patient's consent before they disclose details of their psychotherapeutic treatment to anyone; including those close to the patient." (Notes-Psychotherapies, p.5) With minors this cuts both ways — a parent's legitimate wish to know must be balanced against the adolescent's claim to privacy, and against the special case where "the patient discloses material, which indicates that someone else is (or has been) at risk of harm."

LIMITS OF CONFIDENTIALITY & DUTY TO REPORT

Corey lists the legal requirement to break confidentiality for **child abuse, elder abuse, abuse of dependent adults, and danger to self or others**. With minors, a disclosure of abuse or of serious self-harm risk triggers ***safeguarding*** *(action to protect a child from harm)* and, where mandated, reporting — which *overrides* ordinary confidentiality. The new source confirms this is a legal duty of care: "psychotherapists in the USA are mandated to report suspected child abuse, even when the information comes from their patient, who may be the abuser," while acknowledging the clinician "may feel a conflict between respect for the law, and the avoidance of harm to children."

(Corey, p.77; Notes-Psychotherapies, p.10)

THIRD-PARTY PRESSURE & THE "DON'T TELL THE PATIENT" TRAP

Clinicians "may also come under pressure from others (family members or employers) ... to seek information about the therapy." A specific ethical problem arises "when the third party asks the therapist not to tell the patient about the contact" — common with a parent of an adolescent. The clinician must then "balance the patient's claim to honesty and confidentiality against a possible harm to them." (Notes-Psychotherapies, p.6)

6.5b Consent to use material for research or teaching

A distinct consent is needed before a minor's (or any patient's) material is used for **teaching or research**. The source warns that therapists can no longer "assume that it is ethically unproblematic to use patient material for teaching and research without consent, even if it is disguised," because of "the increased social emphasis on respect for individual patient autonomy." Some journals now require not only consent but that "the patient has *read* the article in which their case is mentioned"; and "getting consent after therapy has ended is not necessarily the answer, as this might be just as intrusive or distressing." (Notes-Psychotherapies, p.6) For minors this means **guardian consent plus the minor's assent** to any teaching/research use, handled separately from consent to treatment.

6.6 Minor–guardian disagreement

Disagreements are common and must be anticipated. Examples: a guardian consents but the adolescent refuses to engage; the adolescent wants therapy a parent opposes; separated parents disagree about treatment; an adolescent discloses something they do not want their parents told. The clinician's task is to (i) clarify the legal position (who holds authority to consent), (ii) work clinically with the minor's reluctance or the family conflict, and (iii) be transparent about the limits of what can be kept private.

6.7 Relevant Indian provisions & professional standards

INDIAN LEGAL CONTEXT — GENERAL KNOWLEDGE; VERIFY AGAINST THE PRIMARY STATUTE

These are stated at a **general** level only. They are *not* drawn from the repository PDFs, and no clause/section numbers are quoted (do not invent them). Always check the current primary statute and your regulator's guidance.

- **Indian Majority Act, 1875** — the age of majority in India is generally **18 years**; below this a person is a minor and a guardian ordinarily consents. *(Statute — verify against the primary text.)*
- **Protection of Children from Sexual Offences (POCSO) Act, 2012** — defines a child (under 18) and creates *mandatory reporting* duties for known or suspected child sexual abuse. *(Statute — verify against the primary text; a clinician who learns of such abuse may be legally obliged to report.)*
- **Mental Healthcare Act (MHCA), 2017** — contains provisions governing the treatment of minors and the role of the nominated representative/guardian. *(Statute — verify against the primary text.)*
- **Digital Personal Data Protection (DPDP) Act, 2023** — treats children's personal data as requiring special protection, generally needing verifiable parental consent for processing. *(Statute — verify against the primary text.)*

Professional standards (e.g. the Rehabilitation Council of India framework for clinical psychologists, and ethics texts such as Corey) reinforce: obtain guardian consent, seek the minor's assent, define confidentiality limits in advance, and document. These are **ethical guidance**, distinct from statute.

CLINICAL–LEGAL FRAMING — CONSENT FAILURES AND NEGLIGENCE

The new source situates consent within malpractice law. "As psychotherapy is a 'talking' treatment, is informed consent necessary? The answer is yes, because all psychotherapies have risks and benefits that patients need to understand." Crucially, "informed consent doctrine provides a basis for legal recovery of compensation for patients who are harmed by failures of therapists to obtain informed consent. Some states that have informed consent statutes declare the **failure to obtain informed consent to be negligence** (*Slovenko, 1989* — cited in Notes-Psychotherapies)." (Notes-Psychotherapies, p.7–8) It also notes "there are over 450 schools of psychotherapy … (*Simon, 2001*)" yet "basic clinical–legal principles generally apply to most, if not all, forms" — so the consent/assent duty is near-universal, not school-specific. (Notes-Psychotherapies, p.7) *(These are binding-law observations about US jurisdictions reported in the source; verify the position in your own jurisdiction.)*

6.8 Clinical application — illustrative examples

ILLUSTRATIVE EXAMPLE — A CHILD (AGE 7)

Illustrative teaching vignette. A 7-year-old is referred for anxiety. The parent gives written **consent**; the clinician obtains the child's **assent** with concrete language ("We'll play some games and talk about scary feelings — is that okay with you?"), involves the parent in the evaluation (cf. Beck's "Involving a Family Member"), and explains to the parent that session content is generally private but safety concerns will be shared.

(Beck-CBT, p.18 [ToC]; Corey, p.77)

ILLUSTRATIVE EXAMPLE — AN ADOLESCENT (AGE 16)

Illustrative teaching vignette. A 16-year-old attends with low mood and asks that her disclosures not be shared with her parents. The clinician negotiates a confidentiality agreement up front: routine content stays private, but **risk of serious self-harm** or **abuse** must be shared/reported. When she later discloses passive suicidal ideation, the clinician applies the safety exception, involves the guardian appropriately, and keeps the therapeutic alliance by having forewarned her of exactly this limit. (Corey, p.77)

6.9 Procedure — steps for consent/assent with a minor

1. Establish **who holds legal authority** to consent (parent/guardian/court order).
2. Obtain **informed consent** from the guardian (goals, methods, risks, confidentiality limits, fees, recording).
3. Obtain the minor's **assent** using developmentally appropriate language.
4. Agree a **confidentiality framework** with both parties, including the exceptions (abuse, danger to self/others).
5. **Document** consent, assent and the confidentiality agreement; if consent is oral, record its nature and extent. (Corey, p.75–76)
6. Revisit as the minor matures or circumstances change.

6.10 Advantages, limitations & challenges

- **Advantage:** the consent+assent model protects the minor legally while honouring their emerging autonomy and improving engagement.
- **Limitation:** shared confidentiality can deter adolescents from full disclosure; the clinician must balance candour and trust.

- **Challenge:** minor–guardian disagreement, custody disputes, and jurisdiction-specific law mean there is no uniform answer — the clinician must reason case by case.

6.11 Mnemonic

MINOR CONSENT & ASSENT

- **M** **Maturity & capacity** assessed (not age alone).
- **I** **Informed consent** from the guardian (legally valid).
- **N** **Negotiate confidentiality** limits with both up front.
- **O** **Obtain the minor's assent** in age-appropriate language.
- **R** **Report & safeguard** — abuse / danger overrides confidentiality; document throughout.

Apply: *Before a first session with a 14-year-old, run MINOR — "N" reminds you to tell both teen and parent that self-harm or abuse disclosures must be shared, so no one is blindsided later.*

6.12 Possible examination questions

1. Distinguish *consent* from *assent* in work with minors, with the roles of guardian and child. (Corey)
2. Why does privileged communication not extend to child and adolescent therapy? (Corey)
3. Discuss confidentiality, its limits and safeguarding duties when the client is a minor, with an adolescent example.
4. "There is no single universal rule for minors." Discuss, referring to law, circumstances and type of service.

EXAM-ORIENTED SUMMARY

- **Consent** (guardian, legally valid) + **assent** (minor, ethically essential) = the two-part model.
- **Valid consent = voluntariness + capacity + information**; capacity is decision-specific & **fluctuates** (children, psychosis, learning disability); the *coerced* and *child-refusal* puzzles (Notes-Psychotherapies p.4–5).
- **Privilege does not extend** to child/adolescent therapy (Corey p.76); **duty to report** child abuse (Corey p.77; Notes-Psychotherapies p.10).
- **Confidentiality = informed consent to disclosure**; negotiate limits up front; safety/abuse overrides it; separate consent for **research/teaching** use (Notes-Psychotherapies p.5–6).
- **Indian law** (Majority Act 1875, POCSO 2012, MHCA 2017, DPDP 2023) — general knowledge; verify against the primary statute; no invented clauses.
- **No universal rule**: depends on law, circumstances, service type. **Mnemonic:** MINOR.

6.13 References

Notes-Psychotherapies p.4, p.5, p.6, p.10 · Corey p.75, p.76, p.77 · Beck-CBT p.18 [ToC]. Secondary authors (H

TOPIC F

7 · Planning & Recording of Therapy Sessions

How a session is planned before it starts and documented after it ends — treatment planning, note formats (SOAP, DAP), documentation principles, recording consent and data handling, plus a worked sample note and a reusable blank template.

7.1 Introduction

Planning and recording are the two bookends of every session. ***Planning*** *(preparing the aims, structure and interventions before/at the session)* keeps therapy coherent and goal-directed; ***recording*** *(documenting what occurred, clinically and legally)* provides continuity of care, accountability, communication with other professionals, and protection for both client and clinician. Documentation is increasingly driven by third-party payors and accrediting agencies demanding fast, auditable records. **(Jongsma, p.14; p.19)**

7.2 Planning a session — the preparation cycle

- **Initial preparation & review of previous sessions.** Before the session, review prior notes, homework and progress against the plan.
- **Case formulation.** Hold a working ***case formulation*** *(a hypothesis linking the client's history, triggers and maintaining factors to their presenting problems)* that the session's work tests and refines.
- **Agenda.** Set a collaborative agenda for what the session will cover (see Topic 8).
- **Intervention selection.** Choose techniques matched to the formulation and stage of therapy.

- **Session closure.** Plan time to summarise, assign between-session tasks and gather feedback.

7.3 Jongsma's six-step treatment plan (canonical)

THE SIX STEPS — JONGSMA

1. **Problem Selection** — identify the primary problem(s) for treatment.
2. **Problem Definition** — specify it behaviourally, tied to DSM-5/ ICD criteria.
3. **Goal Development** — broad, global, long-term; *need not* be measurable.
4. **Objective Construction** — behavioural, measurable/ quantifiable targets.
5. **Intervention Creation** — clinician actions matched to each objective.
6. **Diagnosis Determination** — compare presentation to DSM-5 criteria (needed for third-party reimbursement).

(Jongsma, p.25; p.26)

Plans must be **tailored to the individual** — considering strengths, weaknesses, stressors, social network, family circumstances and symptom patterns — not mass-produced. (Jongsma, p.26) Companion tools (Progress Notes Planners, Homework Planners, Client Education Handout Planners) integrate the plan's definitions and interventions with the record. (Jongsma, p.14; p.19)

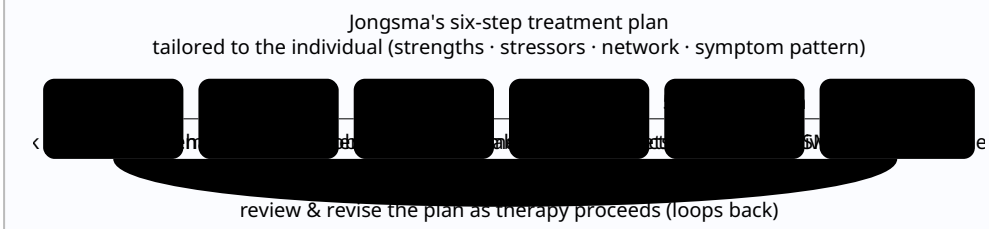


Figure 7.1 — Jongsma's six-step treatment plan. *Problem Selection Problem Definition Goal Development (global, need not be measurable) Objective Construction (measurable) Intervention Creation Diagnosis Determination. The dashed arrow marks that the plan is reviewed and revised, not written once.* ***(Jongsma, p.25–26)***

7.4 Types of record — clinical vs. progress vs. process notes

Table 7.1 — Routine clinical record vs. personal process note vs. research record

Dimension	Routine clinical / progress record	Personal process ("psychotherapy") note	Research record
Purpose	Continuity of care, accountability, billing, legal record	Therapist's private reflections, hypotheses, countertransference	Data for a study / audit of outcomes
Content	Objective facts: presentation, interventions, plan, risk	Impressions, tentative formulations, personal reactions	De-identified measures, scores, protocol adherence
Who may access	Client (on request), supervisors, often third parties with consent	Usually kept separate; narrower access where law allows	Research team; governed by ethics approval & consent
Format	Structured (e.g. SOAP, DAP)	Freeform narrative	Standardised data fields / instruments
Example source	Jongsma Progress Notes; Corey's consent documentation	Wolberg's verbatim recording tradition	Project MATCH outcome data (Topic 9)

Verbatim / process tradition. Wolberg advises that statements of the patient's most important difficulties "should be recorded verbatim if possible," and offers an *initial interview form* that tabulates the interview so the therapist can interact rather than write. (Wolberg-Technique, p.845) **Records & security.** Corsini's ethics vignette (client files on an office computer used for billing) highlights backup, technician access and record-security dilemmas. (Corsini, p.592)

THE "TWO-FILE" PRACTICE — BRIEF MEDICAL NOTE + THEMES FILE

The new source describes a widely-recommended split: "My own practice is to make a **brief note in the medical record** that the session has taken place, and then keep some **brief notes of the main themes** of the session in a file in my office. The American Psychiatric Association recommends this practice." (Notes-Psychotherapies, p.6) The rationale: the therapist keeps a record "to assist ... the process of thinking and supervision," but "generally speaking, the therapist does not share her thoughts and feelings about the patient with the patient" — "it is unlikely that the patient will find it helpful to discover what their therapist thinks about them from simply reading the process notes." (Notes-Psychotherapies, p.6)

PATIENTS' LEGAL ACCESS TO NOTES — THE THERAPIST DOES NOT "OWN" THE NOTES

A crucial honesty point for exams: "legal jurisdictions, both in the UK and in the US, do ... make it theoretically possible for the **patient to have access to those notes**, unless this access would constitute harm to them or another person." Hence "therapists cannot assume that they own the notes of their meetings." (Notes-Psychotherapies, p.6) In **CBT specifically**, "it is common practice for the therapist to record the patient's experiences in therapy (thoughts, feelings, behaviors) ... sharing of these records and notes is often therapeutically helpful." The problem arises when the patient records "information that may be misunderstood out of context," or when the therapist keeps "impressions of the patient ... they do not wish the patient to see; and to which the patient may have legal access." (Notes-Psychotherapies, p.7)

7.5 SOAP and DAP note formats

SOAP and DAP are standard professional documentation formats (general professional knowledge, presented here as such — not attributed to a repository page).

Table 7.2 — SOAP vs. DAP progress-note formats

SOAP	Meaning	DAP	Meaning
Subjective	Client's reported experience, in their words	**D**ata	Subjective + objective combined: what the client reported and what was observed
Objective	Observable facts: mental-state observations, scores, behaviour		
Assessment	Clinical interpretation / formulation of progress	**A**ssessment	Clinician's interpretation of the data
Plan	Next steps, homework, risk actions, next appointment	**P**lan	Next steps and interventions

DAP is essentially SOAP with the S and O merged into a single "Data" field — useful when the subjective/objective split is hard to maintain.

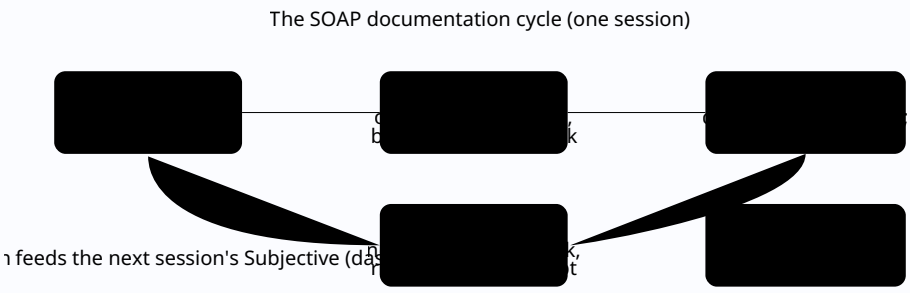


Figure 7.2 — The SOAP note cycle (and the DAP variant). *Each session is documented Subjective Objective Assessment Plan; the Plan then feeds the next session's bridge/subjective, so documentation is a repeating lifecycle, not a one-off. DAP collapses S and O into one "Data" field.*

7.6 Documentation principles

- **Objectivity** — record facts and clearly label interpretations as such.
- **Accuracy** — faithful to what occurred; correct errors transparently.
- **Relevance** — clinically pertinent; avoid gratuitous personal detail.
- **Confidentiality** — secure storage and controlled access.
  (Corsini, p.592)
- **Timeliness** — written promptly after the session while memory is fresh.
- **Documenting consent & ethical decisions.** If consent is oral, "therapists must make an entry in the client's clinical record documenting the nature and extent of informed consent," and document how the client was included in ethical decisions.
  (Corey, p.75; p.76)

7.7 Audio / video recording and data handling

RECORDING CONSENT, STORAGE, ACCESS, RETENTION, DISPOSAL

- **Consent:** obtain specific, informed consent to audio/video recording (for supervision, training or research), separate from consent to therapy itself; for minors, guardian consent + child assent.
- **Storage:** encrypted/secure storage with restricted access.
- **Access:** limited to those with a legitimate need (clinician, supervisor).
- **Retention:** keep only as long as clinically/legally required by policy and law.
- **Disposal:** secure deletion/destruction at the end of the retention period.

Specific retention periods and data-protection duties (e.g. DPDP Act 2023 in India) are law/policy — verify against the primary statute and your institution's policy.

7.8 Practical example — a filled sample session note (illustrative)

ILLUSTRATIVE FILLED SOAP NOTE (FICTIONAL)

Illustrative teaching example — not a real client.

Client ID: AX-021 · **Session:** 4 of planned 12 · **Date:** [DD/MM/YYYY] · **Modality:** CBT, individual, 50 min

S (Subjective): "I managed to go to the supermarket twice this week." Reports panic frequency down from daily to twice this week; still avoids crowded buses.

O (Objective): Alert, oriented, mood euthymic-to-mildly-anxious, affect congruent. Completed thought-record homework (5/7 days). GAD-7 = 11 (down from 15 at intake). No current risk indicators.

A (Assessment): Good early response to graded exposure and cognitive restructuring; avoidance of buses persists as next target. Formulation (catastrophic misinterpretation of bodily sensations) supported.

P (Plan): Build bus-travel exposure hierarchy next session; continue daily thought records; review GAD-7 at session 6. Next appt: [date].

Risk: None identified today. **Clinician:** [name/signature] **Written:** same day.

7.9 A blank reusable template

Reusable blank progress-note template (SOAP; copy & adapt)

BLANK SESSION / PROGRESS NOTE

Client ID: _____ · **Session #:** ____ of ____ · **Date:** __/__/____ · **Duration:** ____ min · **Modality / setting:** _____

S — Subjective: (client's reported experience, in their words) _____

O — Objective: (mental-state observations, scores, homework completion, behaviour) _____

A — Assessment: (clinical interpretation; progress vs. goals; formulation update) _____

P — Plan: (interventions, homework, risk actions, next appointment) _____

Risk screen: (self-harm / harm to others / safeguarding) _____

Consent / recording note: (any consent obtained or updated this session) _____

Clinician: _____ **Signature:** _____ **Date written:** __/__/____

Swap the S/O rows for a single "D — Data" row to convert this into a DAP note.

7.10 Advantages, limitations & challenges

- **Advantage:** structured, timely records support continuity, accountability, supervision and reimbursement.

- **Limitation:** over-documentation can intrude on the relationship (Wolberg's interview form exists precisely to free the therapist to interact). (Wolberg-Technique, p.845)
- **Challenge:** balancing fullness of record with confidentiality and data-security duties. (Corsini, p.592)

7.11 Mnemonic

RECORD DOCUMENTATION

- **R** **Relevant** & clinically pertinent only.
- **E** **Exact / accurate** — facts first, label interpretations.
- **C** **Confidential & secure** storage and access.
- **O** **Objective** before opinion (the S/O before the A).
- **R** **Recording consent** for audio/video, with retention & disposal.
- **D** **Dated & done promptly** (timeliness) and signed.

Apply: *Finishing a note, run RECORD — the final "D" reminds you to write it the same day and sign it, so the record is contemporaneous and defensible.*

7.12 Possible examination questions

1. Describe Jongsma's six-step treatment plan with an example. (Jongsma)
2. Compare SOAP and DAP note formats. When would you prefer each?
3. Distinguish routine clinical records, personal process notes and research records.
4. Outline consent, storage, retention and disposal requirements for audio/video recording of sessions.
5. "Therapists do not own their notes." Discuss patients' legal access to records and its implications for process notes and CBT shared records. (Notes-Psychotherapies)

EXAM-ORIENTED SUMMARY

- **Plan:** review formulation agenda intervention closure; Jongsma's 6 steps (problem definition goal objective intervention diagnosis).
- **Record types:** routine clinical/progress vs. personal process vs. research; the **brief-medical-note + themes-file** split (APA-recommended, per Notes-Psychotherapies p.6).
- **Formats:** SOAP (S/O/A/P); DAP merges S+O into Data.
- **Patient access:** the therapist does *not* "own" the notes — patients may have legal access; CBT often shares records (Notes-Psychotherapies p.6–7).
- **Principles:** objective, accurate, relevant, confidential, timely; document consent (Corey p.75–76).
- **Recording:** consent secure storage limited access retention disposal. **Mnemonic:** RECORD.

7.13 References

Jongsma p.14, p.19, p.25, p.26 · Notes-Psychotherapies p.6, p.7 · Wolberg-Technique p.845 · Corey p.75, p.76

TOPIC G

8 · Structuring Therapy Sessions & Setting Goals

The arc of a whole course of therapy and the anatomy of one session — plus how global goals become SMART, measurable targets, and how structure varies across approaches.

8.1 Introduction

Structuring *(organising therapy — and each session — so time is used purposefully toward agreed goals)* prevents drift, builds the client's sense of progress, and makes outcomes measurable. The degree and type of structure vary sharply between approaches, which is itself a classic exam contrast.

8.2 The arc of therapy — intake to termination

A widely taught sequence (integrating Jongsma's planning and Beck's session model):

1. **Intake** — referral, consent, presenting problem.
2. **Assessment** — history, measures, risk (see Topic 9).
3. **Formulation** — a working hypothesis linking factors to problems.
4. **Treatment planning** — goals + measurable objectives + interventions (Jongsma's six steps). (Jongsma, p.25; p.26)
5. **Intervention** — the main body of therapy.
6. **Review** — monitor progress; revise the plan.
7. **Termination** — consolidate gains, relapse prevention, ending.

8.3 Anatomy of a single session (CBT model — Judith Beck)

Beck sets out an explicit structure. The **first therapy session**: set the agenda mood check obtain an update discuss the diagnosis **problem identification and goal setting** educate the patient about

the cognitive model behavioural activation **end-of-session summary and homework** feedback. (Beck-CBT, p.18 [ToC])

Session 2 and beyond has a repeating structure: *first part* (mood check, brief update, bridge from last session, review homework, set agenda) *middle part* (discuss agenda items, set new homework) *final* (summary and feedback). Beck devotes a chapter to *"problems with structuring the session"* (therapist cognitions about structure, interrupting the patient, strengthening the alliance).

(Beck-CBT, p.18 [ToC]; p.19 [ToC])

THE SIX BEATS OF A STRUCTURED SESSION

1. **Rapport / mood check** — reconnect, brief mood rating.
2. **Review** — bridge from last session; review homework.
3. **Set agenda** — collaboratively prioritise today's items.
4. **Interventions** — work the agenda.
5. **Summarise** — capstone summary, check understanding.
6. **Between-session tasks** — assign homework; gather feedback.

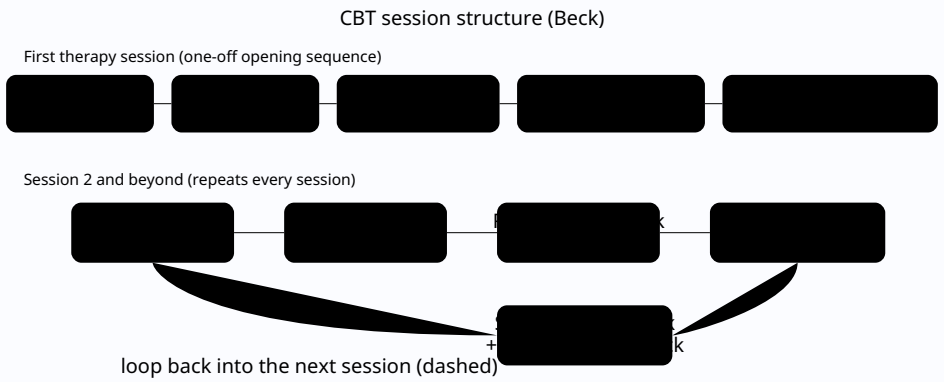


Figure 8.1 — CBT session structure. *The first session runs a one-off opening sequence (agenda/mood update/diagnosis problems + goal setting educate + behavioural activation summary + homework + feedback). From session 2 onward a tight loop repeats: mood check bridge review homework + agenda work agenda items set new homework summary/feedback, feeding into the next session.* ***(Beck-CBT, p.18–19 [ToC])***

8.4 Goals — short-term vs. long-term (Jongsma)

Table 8.1 — Long-term goals vs. short-term objectives

Feature	Long-term goal	Short-term objective
Scope	Broad, global, directional	Specific, narrow step
Measurability	Need *not* be measurable	Behavioural, measurable/ quantifiable
Example	"Reduce overall level of anxiety"	"Attend 3 feared situations this week and rate anxiety 0–10"
Role	States the desired positive outcome	Lets attainment be measured and tracked

(Jongsma, p.24; p.25)

8.5 SMART goals and collaborative goal-setting

The ***SMART*** framework (general professional knowledge) turns a vague aim into a workable objective:

SMART GOAL FRAMEWORK

- **S** **Specific** — clearly defined, not vague.
- **M** **Measurable** — progress can be quantified.
- **A** **Achievable** — realistic for this client now.
- **R** **Relevant** — tied to the client's values & the long-term goal.
- **T** **Time-bound** — has a timeframe for review.

Apply: *A client says "I want to be happier." Run SMART to co-write: "Walk with a friend twice a week for 4 weeks, then review mood on the PHQ-9" — specific, measurable, achievable, relevant, time-bound.*

Goal-setting is **collaborative**: the therapist discusses goals, length and methods early and develops them *with* the client, avoiding over-responsibility for outcomes. (Corey, p.63) Goals are then **prioritised**, **monitored** and **revised** as therapy proceeds. The new source grounds this in ethics: psychotherapy should pursue **beneficence and nonmaleficence**, yet "there may still be uncertainty or conflict about what the goals of therapy are, and who should set them, which can make the issue of informed consent in psychotherapy particularly complex" — so the clinician should "warn patients that the process of psychotherapy is not always comfortable, and that many patients feel worse before they feel better (although this may not be true for CBT)." (Notes-Psychotherapies, p.4)

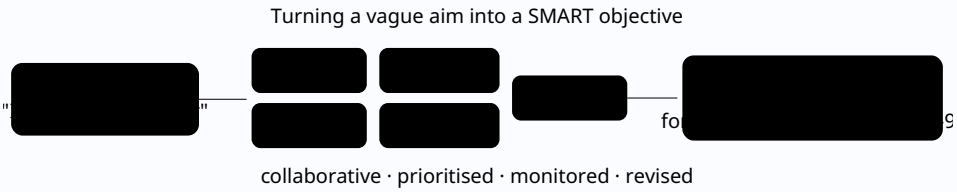


Figure 8.2 — The SMART goal-setting mini-flow. *A vague aim is passed through the five SMART filters (Specific, Measurable, Achievable, Relevant, Time-bound) to yield a concrete, trackable objective — co-written with the client and revised as therapy proceeds.*

8.6 Converting vague goals into measurable clinical goals

CONVERSION EXAMPLE 1 (ILLUSTRATIVE)

Vague goal: "I want to feel less depressed." *Measurable goal:* "Reduce PHQ-9 score from 18 to below 10 within 8 weeks; engage in at least two pleasant activities per day logged on an activity schedule."

CONVERSION EXAMPLE 2 (ILLUSTRATIVE)

Vague goal: "I want to be more confident socially." *Measurable goal:* "Initiate one conversation with a colleague each workday for 3 weeks and rate anxiety 0–10 before/after; attend one social event by week 6."

CONVERSION EXAMPLE 3 (ILLUSTRATIVE)

Vague goal: "I want to stop arguing with my partner." *Measurable goal:* "Use a time-out strategy during conflicts; reduce escalated arguments from ~5/week to 1/week over 6 weeks, tracked in a shared log."

8.7 How structure varies across approaches (key exam point)

Table 8.2 — Structured vs. less-directive approaches

Dimension	CBT (structured)	Psychodynamic (less directive)
Agenda	Explicit, collaborative agenda each session (Beck-CBT, p.18)	Open; free-association oriented, no set agenda
Homework	Routine between-session tasks	Rarely assigned
Therapist stance	Active, directive, psycho-educational	More receptive; interprets rather than directs
Structure comes from	Agenda, goals, homework	The ***frame*** *(consistent time, place, boundaries)* — e.g. a therapist checking the time may be *experienced* by the patient as a slight, to be explored, not just managed (Gabbard, p.64)
What shapes it	Protocol / formulation	Patient's level of organisation on an exploratory supportive continuum (Gabbard, p.46)

Structure also varies by *context*: crisis/brief work is tightly focused; long-term depth work is more open; group and family formats add their own structure.

8.8 Advantages, limitations & challenges

- **Advantage:** structure and measurable goals create direction, momentum and a basis for outcome measurement.
- **Limitation:** rigid structure can rupture the alliance or miss what the client most needs to raise — Beck explicitly flags "problems with structuring." (Beck-CBT, p.19 [ToC])
- **Challenge:** matching the degree of structure to the client, approach and phase of therapy.

8.9 Mnemonic

RASISB SESSION ANATOMY

- **R** **Rapport / mood check.**
- **A** **Agenda** set collaboratively.
- **S** **Status review** (bridge + homework review).
- **I** **Interventions** on the agenda items.
- **S** **Summarise** and check understanding.
- **B** **Between-session tasks** + feedback.

Apply: *Planning a CBT session, run RASISB — it maps directly onto Beck's structure, ensuring you reserve time for the summary and homework rather than overrunning on one agenda item.*

8.10 Possible examination questions

1. Describe the structure of a CBT session from start to finish. **(Beck)**
2. Differentiate long-term goals from short-term objectives, with examples. **(Jongsma)**
3. Convert two vague client goals into SMART, measurable clinical goals.
4. Compare how structure differs between CBT and psychodynamic therapy. **(Beck; Gabbard)**

EXAM-ORIENTED SUMMARY

- **Arc:** intake assessment formulation plan intervention review termination.
- **Session beats:** rapport review agenda interventions summary homework (Beck).
- **Goals:** long-term (global, need not be measurable) vs. short-term objectives (measurable); use SMART.
- **Structure varies:** CBT structured + agenda + homework; psychodynamic less directive, structured by the "frame" (Gabbard).
- **Mnemonics:** RASISB (session), SMART (goals).

8.11 References

Beck-CBT p.18, p.19 [ToC] · Jongsma p.24, p.25, p.26 · Gabbard p.46, p.64 · Corey p.63 · Notes-Psychotherapie

TOPIC H

9 · Pre- and Post-Assessment

Measuring where the client starts, tracking change, and judging whether it is real and meaningful — baseline, ongoing monitoring, post-treatment and follow-up, with the methods, psychometric criteria, and the limits of a single test.

9.1 Introduction

Pre- and post-assessment *(measuring the client's status before, during and after treatment to establish a baseline and to detect change)* turns subjective impressions of "improvement" into something that can be observed, compared and acted on. It feeds treatment planning, detects non-response or deterioration early, and contributes to the evidence base.

9.2 The four time-points

- **Baseline (pre-treatment)** — the reference point before therapy begins.
- **Ongoing monitoring (during)** — repeated measurement to track trajectory.
- **Post-treatment** — status at the end of therapy.
- **Follow-up** — later re-assessment to check maintenance of gains.

Model from the manuals. In Motivational Enhancement Therapy the first two sessions "focus on structured feedback from the initial assessment," and in Project MATCH clients were "evaluated at intake and again at 3, 6, 9, 12, and 15 months" — a template of baseline plus repeated follow-up on drinking behaviour, psychological/social function and consequences. (MET, p.11) The new source underscores why accurate assessment matters: "successful psychotherapy matches the right therapy and right therapist to the patient, which is why therapists need to develop good quality assessment skills" — including the ability "to formulate problems in different ways." (Notes-Psychotherapies, p.4)

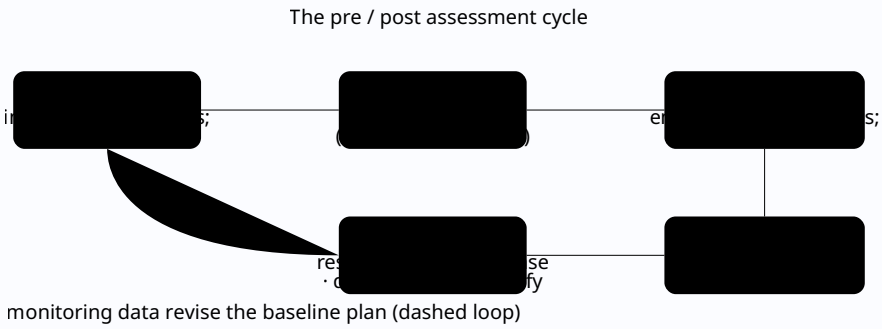


Figure 9.1 — The pre/post assessment cycle. *Baseline (PRE) treatment with ongoing monitoring post/outcome measurement follow-up. The data feed back into the treatment plan (response, non-response, deterioration modify), mirroring Project MATCH's intake + 3/6/9/12/15-month re-assessment.* ***(MET, p.11)***

9.3 Methods of assessment

- **Clinical interview** — the core method; history, presenting problem, risk.
- **Behavioural observation** — directly observing behaviour in session or in vivo.
- **Standardised measures** — validated questionnaires/rating scales (e.g. symptom scales). Instrument *types* such as the MCMI, Rorschach, TAT, MISIC or VSMS may serve as pre/post examples, but their content is not described here because those manuals are image-only scans and were not reviewed.
- **Functional assessment** — analysing antecedents, behaviour and consequences to understand what maintains a problem.
- **Client-reported outcomes** — the client's own ratings of symptoms, functioning and goal attainment. Routine systems such as CORE collect such data in practice. **(Lister-Ford, p.118)**

9.4 Psychometric criteria for a good outcome measure

Table 9.1 — What makes a measure fit for pre/post use

Criterion	Meaning	Why it matters for pre/post
Reliability	Consistency of measurement (e.g. test–retest)	Change must reflect the client, not measurement noise
Validity	Measures what it claims to measure	Otherwise "improvement" may be on an irrelevant construct
Sensitivity to change	Able to detect real improvement/worsening over time	A stable "trait" measure may miss genuine change
Clinical significance	Change that is meaningful, not just statistically detectable	Moving into the non-clinical range matters more than a few points
Meaningful functional improvement	Change shows up in the client's real-life functioning	The ultimate test of a worthwhile outcome

9.5 Selecting measures

Choose instruments by **client** (age, literacy), **language** (validated translation), **population** (norms that fit), **purpose** (screening vs. outcome vs. diagnosis), and **evidence** (established reliability/validity for this use). A measure validated for one group may not be valid for another.

9.6 Outcome monitoring — response, non-response, deterioration

- **Response** — meaningful improvement on repeated measurement continue/consolidate.
- **Non-response** — no improvement by the expected point review formulation, dose, fit, alliance; modify treatment.

- **Deterioration** — worsening scores urgent review, risk check, possible change of plan or referral.

Monitoring data also inform the therapist's own development; CORE is used to monitor effectiveness and flag CPD needs. (Lister-Ford, p.118)

9.7 Why a single test does not establish treatment effectiveness

THE SINGLE-TEST CAVEAT (IMPORTANT)

Improvement on one test from pre to post does **not**, by itself, prove the *therapy* caused the change. Rival explanations include: ***spontaneous remission*** *(natural recovery over time)*, ***regression to the mean*** *(extreme baseline scores drifting toward average on re-test)*, ***measurement error***, practice/placebo effects, and life events. Establishing *effectiveness* requires controlled comparison, replication and multiple convergent measures — which is why outcome research uses control groups and meta-analysis (Topic 10), not a single pre/post score. Smith & Glass's (1977) meta-analysis of 475 studies found the average treated patient better off than ~80% of untreated controls — a conclusion that depended on *aggregated controlled* data, not one test.

(Lister-Ford, p.62)

9.8 Worked example — pre / during / post / follow-up (illustrative)

ILLUSTRATIVE WORKED EXAMPLE (FICTIONAL)

- **Pre (baseline):** Client with depression; PHQ-9 = 19 (moderately severe), WSAS (functioning) = 24. Clinical interview + client-reported outcomes gathered; treatment plan agreed.
- **During (monitoring):** PHQ-9 at sessions 3/6/9 = 16 12 9, showing a response trajectory; session-by-session feedback confirms engagement.
- **Post (end of treatment):** PHQ-9 = 6 (below clinical threshold), WSAS = 11 — change is both statistically and *clinically* significant (crossed into the non-clinical range) and shows functional improvement (returned to work part-time).
- **Follow-up (3 & 6 months):** PHQ-9 = 7 then 6 — gains maintained, mirroring the MET/Project MATCH repeated-follow-up logic. (MET, p.11)
- **Interpretation caveat:** this pattern is *consistent with* benefit, but on its own cannot rule out other causes; convergent measures + the broader evidence base support (not prove) effectiveness.

9.9 Advantages, limitations & challenges

- **Advantage:** systematic measurement catches non-response/ deterioration early and demonstrates meaningful change.
- **Limitation:** a single measure at two time-points cannot establish causation; measures can lack sensitivity or cultural validity.
- **Challenge:** selecting valid, change-sensitive, culturally appropriate measures and interpreting change as *clinically* (not just statistically) significant.

9.10 Mnemonic

BMPF ASSESSMENT TIME-POINTS

- **B** **Baseline** before therapy.
- **M** **Monitor** during (repeated measures).
- **P** **Post**-treatment status.
- **F** **Follow-up** to check maintenance.

Apply: *Planning outcome measurement, run BMPF — "F" reminds you to schedule a 3-month follow-up so you know whether gains held, exactly as Project MATCH re-assessed at 3/6/9/12/15 months (MET p.11).*

9.11 Possible examination questions

1. Describe the role of pre-, during-, post- and follow-up assessment, with a worked example. (MET)
2. What psychometric properties make a measure suitable for outcome monitoring?
3. Explain why improvement on a single test does not establish treatment effectiveness. (Lister-Ford)
4. How do you respond clinically to response, non-response and deterioration?

EXAM-ORIENTED SUMMARY

- **Time-points:** baseline monitor post follow-up (MET / Project MATCH 3–15 months).
- **Methods:** interview, observation, standardised measures, functional assessment, client-reported outcomes.
- **Criteria:** reliability, validity, sensitivity to change, clinical significance, functional improvement.
- **Single-test caveat:** pre/post change ≠ proof of effectiveness (spontaneous remission, regression to the mean, error); need controlled, replicated, convergent data.
- **Mnemonic:** BMPF.

9.12 References

MET p.11 · Lister-Ford p.62, p.118 · Notes-Psychotherapies p.4 (assessment-skill rationale) · Hofmann-ArtSci

TOPIC I/J

10 · Practice of Evidence-Based Therapies

What evidence-based practice actually means — the integration of research, expertise and client values — why that differs from merely using a therapy "labelled" evidence-based, and how to select, implement, monitor and modify treatment responsibly.

10.1 Introduction

The push for ***evidence-based practice (EBP)*** reshaped the field: clinicians are expected to justify what they do with research while still tailoring it to the individual. But EBP is frequently misunderstood as "just use a brand-name therapy that has been researched" — a narrowing that the definition below explicitly resists.

10.2 Definition — the three-pillar model (APA 2006 style)

EBP AS INTEGRATION (APA 2006 DEFINITION — GENERAL KNOWLEDGE, LABELLED)

Evidence-based practice in psychology is the **integration of (1) the best available research evidence with (2) clinical expertise in the context of (3) patient characteristics, culture, preferences and values.** *(This three-pillar formulation is the American Psychological Association's 2006 definition, stated here as general professional knowledge — not drawn from a repository page.)*

The crucial point: EBP is a *process of integration and clinical decision-making*, not a product. All three pillars are required; research evidence alone is necessary but not sufficient.

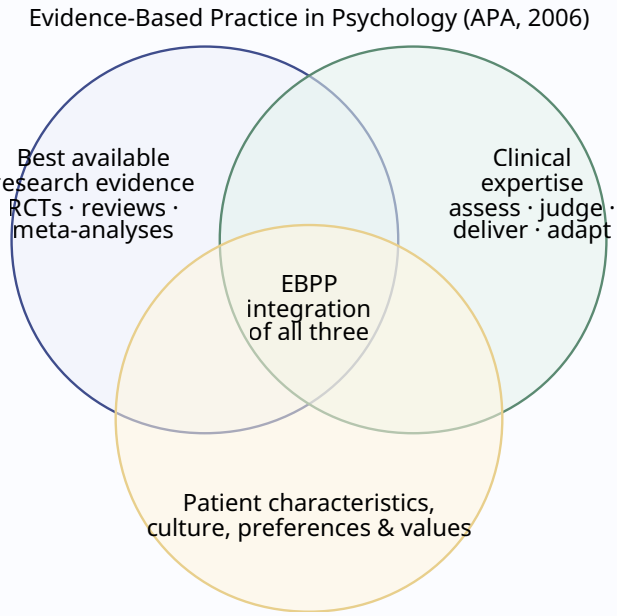


Figure 10.1 — The EBP three-circle Venn (APA, 2006). *Evidence-based practice in psychology is the integration at the intersection of (1) the best available research evidence, (2) clinical expertise, and (3) patient characteristics, culture, preferences and values. No single circle is sufficient — EBPP lives where all three overlap.*
(APA Presidential Task Force on Evidence-Based Practice, 2006 — general knowledge)

10.3 EBP vs. an "evidence-based" brand therapy (key distinction)

Table 10.1 — Doing EBP vs. merely using an "evidence-based" therapy

Aspect	Evidence-based *practice* (the process)	Using an "evidence-based" *brand* therapy
What it is	Integrating research + expertise + client values for *this* client	Delivering a named protocol because it has research support
Client fit	Explicitly weighed (culture, preferences, comorbidity)	May be applied generically, regardless of fit
Clinical judgement	Central — selects, adapts, monitors, modifies	May be minimal if the manual is followed rigidly
Risk	—	Assuming a therapy "works for everyone," ignoring individual differences

DO NOT CLAIM A THERAPY "WORKS EQUALLY FOR EVERYONE"

An EST having research support in trials does **not** mean it works equally for every client, culture or condition. EBP requires judging fit and monitoring response — not assuming universal efficacy.

10.4 Empirically supported treatments (ESTs) & the RCT "gold standard"

The 1995 APA **Division 12 Task Force** report (*Training in and Dissemination of Empirically-Validated Psychological Treatments*, Chambless et al.) produced a preliminary list of ***Empirically Validated Therapies*** (EVT, later "supported", ***EST***) with hierarchical research-quality criteria; the ***RCT*** *(randomised controlled trial — participants randomly allocated to treatment vs. a control condition)*, borrowed from psychopharmacology, became the apparent "gold standard." Hofmann's authors critique this "methodcentric reasoning."

(Hofmann-ArtScience, p.53)

10.5 Research quality — RCTs, reviews, meta-analyses, effect sizes

Table 10.2 — Levels of outcome evidence

Method	What it does	Strength / limitation
RCT	Randomises clients to treatment vs. control (waiting-list/placebo); uses manuals/protocols	Strong internal validity; may have limited generalisability to routine practice
Systematic review	Structured synthesis of all eligible studies	Reduces selective citation; depends on study quality
Meta-analysis	Statistically combines many studies into a pooled ***effect size*** *(a standardised measure of how large an effect is)*	Smith & Glass (1977): avg treated patient better off than ~80% of untreated; Lambert et al. (1986) ~70% vs ~40% (Lister-Ford, p.62)

RCT in practice (example). The NIMH Treatment of Depression Collaborative Research Program compared CBT vs. IPT vs. two non-psychotherapy conditions. (Lister-Ford, p.64) **Efficacy vs. effectiveness.** Efficacy studies (RCTs) test treatment under controlled conditions; effectiveness/"practice-based evidence" tests it in routine settings (CORE, Practice Research Networks). "No evidence of effectiveness" is not "evidence of ineffectiveness" (Roth & Fonagy, 1996).
(Lister-Ford, p.118)

10.6 Manuals, guidelines, fidelity, adaptation, competence

- **Treatment manuals** specify the protocol so it can be delivered consistently and studied.
- **Fidelity** *(the degree to which a treatment is delivered as intended)* is measured by tools such as the Cognitive Therapy Rating Scale (CTRS), used to assess therapist competence in research and training. (Beck-CBT, p.16)

- **Adaptation & competence** — manuals must be applied with skill and adapted to the client, balancing fidelity with responsiveness.
- **Guidelines controversy.** Corsini: proponents want standardisation; critics reject "cookbook" protocols; many "guidelines" are actually *expert-consensus* reports (Shaneyfelt & Centor, 2009); there is "no general agreement about the definition, methodology, or value of evidence-based psychotherapy." Each chapter examines the evidence base and "the current controversy regarding the importance — and limitations — of evidence-based practice." (Corsini, p.592; p.16)

10.7 Common factors & integration

Wolberg's 506-Questions notes that ~**two-thirds of all patients improve irrespective of type** of therapy, with little superiority of any single method — raising whether specific techniques are "epiphenomena" secondary to ***common factors*** *(ingredients shared across therapies, e.g. the alliance, expectation, a credible rationale)*. (Wolberg-506Q, p.16) Norcross's four routes to integration:

Table 10.3 — Norcross's four routes to integration; Eclecticism vs. Integration

Route	Core idea
Technical eclecticism	Use procedures from different sources *without* adopting their theories (e.g. Lazarus's multimodal therapy; Beutler's STS)
Theoretical integration	Blend two or more theories into a new framework
Common factors	Identify and use the core ingredients shared across therapies
Assimilative integration	Work from one theory while incorporating techniques from others

Norcross's Table 1.1 contrasts **Eclecticism** (divergent, choosing, atheoretical-but-empirical, "sum of the parts") with **Integration**

(convergent, combining, more theoretical, "more than the sum of the parts"). (Norcross-Handbook, p.29; p.30)

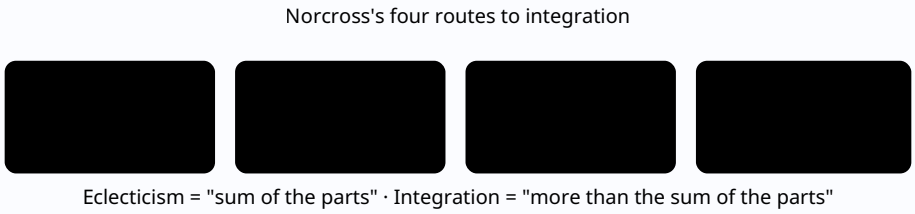


Figure 10.2 — Norcross's four routes to integration. *Technical eclecticism (procedures without their theories), theoretical integration (blend theories), common factors (shared ingredients), and assimilative integration (one home theory plus borrowed techniques). (Norcross-Handbook, p.29–30)*

10.8 The responsible EBP cycle — select implement monitor modify

1. **Select** — choose an approach supported by evidence *and* suited to this client's characteristics, culture and preferences.
2. **Implement** — deliver competently, with adequate fidelity, adapting sensibly.
3. **Monitor** — use pre/post and ongoing measures (Topic 9) to track response.
4. **Modify** — if non-response/deterioration, revise the plan, approach or referral.

ILLUSTRATIVE DECISION-MAKING EXAMPLE

Illustrative teaching vignette. A clinician considering an EST for panic weighs the RCT evidence (research), their own skill in delivering it (expertise), and the client's preference to avoid interoceptive exposure plus a cultural factor affecting homework (client values). They begin the protocol, monitor with a panic diary + measure, and when progress stalls, adapt the pacing rather than abandoning or rigidly continuing — EBP as integration, not protocol-worship.

10.9 Advantages, limitations & challenges

- **Advantage:** grounds practice in the best evidence while keeping the client central; improves accountability.
- **Limitation:** RCT evidence may not generalise to routine, comorbid or culturally diverse clients ("methodcentric" critique). (Hofmann-ArtScience, p.53)
- **Challenge:** balancing fidelity with adaptation; the unresolved controversy over guidelines and the role of common factors. (Corsini, p.592; Wolberg-506Q, p.16)

10.10 Mnemonic

PIER EBP PILLARS + CYCLE

P **Patient** characteristics, culture, preferences, values (pillar 3).

I **Integrate** — EBP is integration, not one pillar alone.

E **Evidence** (best available research) + **Expertise** (clinical skill) (pillars 1 & 2).

R **Review** & revise — monitor response and modify.

Apply: *Choosing a treatment, run PIER — the "P" stops you assuming an evidence-based brand "works for everyone" and forces you to weigh this client's culture and preferences.*

10.11 Possible examination questions

1. Define evidence-based practice and distinguish it from merely using an "evidence-based" therapy. (APA 2006 style)
2. Discuss the EST movement and the RCT "gold standard," with its critique. (Hofmann)
3. Compare RCTs, systematic reviews and meta-analyses as sources of evidence.

4. Describe Norcross's four routes to integration and the eclecticism–integration distinction. (Norcross)
5. "Two-thirds improve irrespective of type." Discuss common factors vs. specific techniques. (Wolberg-506Q)

EXAM-ORIENTED SUMMARY

- **EBP = three pillars:** best research evidence + clinical expertise + patient characteristics/culture/values (APA 2006).
- **EBP ≠ brand:** it is a process of integration, not just delivering a researched protocol; never "works for everyone."
- **Evidence levels:** RCT (Div. 12, 1995; "gold standard" + critique) systematic review meta-analysis + effect size (Smith & Glass ~80%).
- **Manuals/fidelity:** CTRS; guidelines controversy (Corsini); common factors (Wolberg ~2/3 improve).
- **Integration:** technical eclecticism, theoretical integration, common factors, assimilative (Norcross). **Cycle:** select implement monitor modify. **Mnemonic:** PIER.

10.12 References

Hofmann-ArtScience p.53 · Corsini p.16, p.592 · Lister-Ford p.62, p.64, p.118 · Norcross-Handbook p.29, p.30 ·

REVISION

Unit I Mnemonic Master Sheet

Every mnemonic in these notes in one place — letter ↔ concept mapping plus a one-line usage example. Use this as a last-minute consolidation sheet.

TRIP-E

T1 · DEFINITION

T **Trained** person · **R** **Relationship** (professional) · **I** **Intentional objectives** · **P** **Psychological means** · **E** **Emotional problems**.

Use: *"Is X psychotherapy?" — a 5-minute medication visit fails P and R, so it is treatment but not psychotherapy.*

SMP-G

T2 · OBJECTIVES

S **Symptom** removal/modification · **M** **Mediate** disturbed behaviour patterns · **P** **Personality Growth**.

Use: *Asked for Wolberg's aims, SMP-G recovers all three in order.*

SUPER-V

T3 · TRAINING

S **Self-awareness** · **U** **Use of self** · **P** **Personal therapy** · **E** **Education (theory)** · **R** **Reflection** · **V** **superVision** (+ CPD).

Use: *Listing pillars of therapist development, SUPER-V covers the "chief instrument is the self" plus supervision/CPD.*

IDENTIFY T4 · ETHICAL DECISIONS

- Identify problem · Determine relevant codes · Explore principles · Note possible actions · Transparency/consult · Include client · Form a plan · Yield & document / review.

Use: *Faced with an ethical dilemma, IDENTIFY gives a defensible, documentable sequence (consistent with Corey).*

RIGHTS T5 · RIGHTS & RESPONSIBILITIES

- Right to information · Informed, autonomous decisions · Goals/methods disclosed · How responsibility is shared · Transparency about supervision · Safety & competence.

Use: *At intake, RIGHTS scripts your opening contract with the client.*

MINOR T6 · CONSENT & ASSENT

- Maturity/capacity · Informed consent (guardian) · Negotiate confidentiality limits · Obtain the minor's assent · Report & safeguard; document.

Use: *Before a first session with a 14-year-old, MINOR ensures teen and parent both know the safety limits in advance.*

RECORD T7 · DOCUMENTATION

- Relevant · Exact/accurate · Confidential & secure · Objective before opinion · Recording consent (retention/disposal) · Dated & done promptly, signed.

Use: *Finishing a note, RECORD's final "D" reminds you to write it the same day and sign it.*

RASISB T8 · SESSION ANATOMY

R Rapport/mood check · Agenda · Status review (homework) · Interventions · Summarise · Between-session tasks + feedback.

Use: *RASISB maps onto Beck's structure so you reserve time for the summary and homework.*

SMART T8 · GOALS

SM Specific · Measurable · Achievable · Relevant · Time-bound.

Use: *Turn "I want to be happier" into "walk with a friend twice weekly for 4 weeks, then review PHQ-9."*

BMPF T9 · ASSESSMENT POINTS

BM Baseline · Monitor (during) · Post-treatment · Follow-up.

Use: *BMPF's "F" reminds you to schedule follow-up, as Project MATCH did at 3/6/9/12/15 months.*

PIER T10 · EBP

R Patient characteristics/culture/values · Integrate (not one pillar alone) · Evidence + Expertise · Review & revise.

Use: *PIER's "P" stops you assuming an evidence-based brand "works for everyone."*

ONE-GLANCE INDEX

T1 **TRIP-E** · T2 **SMP-G** · T3 **SUPER-V** · T4 **IDENTIFY** · T5 **RIGHTS** · T6 **MINOR** · T7 **RECORD** · T8 **RASISB** + **SMART** · T9 **BMPF** · T10 **PIER**.

REVISION

Examination Preparation (All 10 Topics)

A consolidated exam bank spanning every Unit I topic: key terms, short- and long-answer, compare-and-contrast, clinical/ethical dilemmas, case-based questions with answer frameworks, high-yield tables, common confusions, and a rapid-revision sheet.

NOTE ON THESE QUESTIONS

All questions below are offered as **practice / likely revision** items to structure study. They are *not* predictions that any specific question will appear in an examination.

E.1 Key definitions & terms

Table E.1 — High-yield definitions

Term	One-line definition	Topic
Psychotherapy	Treatment, by psychological means, of emotional problems by a trained person in a deliberate professional relationship (Wolberg)	1
Therapeutic alliance	The collaborative affective bond between therapist and client	1,3
Informed consent	Ethical/legal authorisation to proceed, broader than a signed form (Corey)	4,6
Privileged communication	Legal protection of confidential communications from court disclosure; not applicable to child/ adolescent therapy	4,6
Consent vs. assent	Guardian's legal authorisation vs. the minor's developmentally appropriate agreement	6
Capacity / competence	Ability to understand, weigh and communicate a choice; decision-specific and may fluctuate	6
Consent to disclosure	Confidentiality reframed as an ethical duty to get consent before disclosing (Notes-Psychotherapies)	6,7
SOAP / DAP	Progress-note formats: Subjective-Objective-Assessment-Plan / Data-Assessment-Plan	7
SMART goal	Specific, Measurable, Achievable, Relevant, Time-bound objective	8
Clinical significance	Change that is meaningful in real life, not merely statistically detectable	9
Evidence-based practice	Integration of best research evidence + clinical expertise + patient values (APA 2006)	10
Fidelity	Degree to which a treatment is delivered as intended (e.g. CTRS)	10

E.2 Short-answer questions (practice)

1. State Wolberg's three objectives of psychotherapy. (T2)
2. Distinguish confidentiality from privileged communication. (T4)
3. Define consent and assent in work with minors. (T6)
4. State the three legal ingredients of valid consent and name groups whose capacity may fluctuate. (T6)
5. List the components of a SOAP note. (T7)
6. What are the five SMART criteria? (T8)
7. Name the four time-points of outcome assessment. (T9)
8. State the three pillars of evidence-based practice. (T10)

E.3 Long-answer / essay questions (practice)

1. "Psychotherapy is one of the most ambiguous terms in mental health." Discuss the problem of definition and give a working definition. (T1)
2. Discuss informed consent as an ethical and legal foundation of practice, including its elements and the limits of confidentiality. (T4,T5)
3. Describe treatment planning and documentation in psychotherapy, covering Jongsma's six steps, note formats, and recording of consent. (T7)
4. Critically discuss evidence-based practice: its definition, the EST/RCT movement, and the controversy over guidelines and common factors. (T10)

E.4 Compare-and-contrast questions (practice)

1. Consent vs. assent (roles, legal status, language). (T6)
2. SOAP vs. DAP vs. personal process notes. (T7)
3. CBT (structured) vs. psychodynamic (less directive) session structure. (T8)
4. Efficacy vs. effectiveness vs. practice-based evidence. (T9,T10)

5. Eclecticism vs. integration (Norcross Table 1.1). (T10)

E.5 Clinical-application & ethical-dilemma questions (practice)

1. A 16-year-old asks you not to tell her parents about her self-harm. How do you proceed? (T6)
2. You wish to video-record sessions for supervision. What consent and data-handling steps are required? (T7)
3. You want to present an adolescent client's case at a teaching seminar. What consent (and assent) must you obtain, and why is post-therapy consent not a simple fix? (T6)
4. A client's symptom score worsens at mid-treatment. How do you respond? (T9)
5. You are urged to apply a manualised EST rigidly to every client. Argue for an EBP (integration) stance instead. (T10)

E.6 Case-based questions with model answers / frameworks (practice)

Case 1 — Minor with disclosed abuse (T6)

Case. A 12-year-old in therapy (parent consented, child assented) discloses physical abuse by a relative.

ANSWER FRAMEWORK

1. **Recall the limit:** abuse of a minor is a legal exception to confidentiality and may trigger mandatory reporting (Corey p.77; POCSO 2012 — verify against the statute).
2. **Act:** follow safeguarding policy; report to the designated authority; prioritise the child's safety.
3. **Alliance:** because the confidentiality limit was explained at the outset (part of assent), the child was forewarned; handle the disclosure supportively.
4. **Document:** record the disclosure, actions and rationale contemporaneously.

Case 2 — Outcome monitoring and non-response (T9)

Case. A client with depression shows PHQ-9 of 18 17 18 across six sessions of CBT.

ANSWER FRAMEWORK

1. **Label it:** this is non-response (no meaningful change by the expected point).
2. **Review:** re-check formulation, fidelity/competence (CTRS), alliance, comorbidity, adherence to homework, and external stressors.
3. **Modify:** adjust the approach, intensify, add a component, or consider referral/medication review.
4. **Caveat:** interpret scores alongside clinical judgement — a single measure is not the whole picture (single-test caveat).

Case 3 — Choosing a treatment (EBP) (T10)

Case. Two therapies have RCT support for the client's condition, but one conflicts with the client's cultural preferences.

ANSWER FRAMEWORK (PIER)

1. **Evidence:** both have research support — necessary but not sufficient.
2. **Expertise:** consider which you can deliver competently.
3. **Patient values:** weigh culture/preferences — the three-pillar model requires it; avoid "works for everyone."
4. **Review:** select the better-fitting option, monitor response, and modify if needed.

E.7 Common conceptual confusions & how to avoid them

DON'T MIX THESE UP

- **Confidentiality ≠ privilege.** Confidentiality is ethical/clinical; privilege is a legal court-protection (and does not cover child/adolescent therapy).
- **Consent ≠ assent.** Consent is the guardian's legally valid authorisation; assent is the minor's ethical agreement.
- **Goal ≠ objective.** Long-term goals are global and need not be measurable; short-term objectives must be measurable (Jongsma).
- **Statistical ≠ clinical significance.** A detectable change may still be trivial in real life.
- **Pre/post change ≠ proof of effectiveness.** Rule out spontaneous remission, regression to the mean, measurement error.
- **EBP ≠ a brand therapy.** EBP is integration of three pillars, not merely delivering a researched protocol.
- **Efficacy ≠ effectiveness.** Efficacy = controlled (RCT) conditions; effectiveness = routine practice.

E.8 Rapid-revision sheet — all 10 topics

Table E.2 — One-line recall per topic

#	Topic	Must-recall	Mnemonic
1	Definitions	Wolberg's working definition; ambiguity (Boulder); operational vs understanding-based	TRIP-E
2	Objectives	Symptom change + pattern change + growth; collaborative, measurable objectives	SMP-G
3	Training	Self as instrument; personal therapy, supervision, CPD; CTRS competence	SUPER-V
4	Ethics & law	Informed consent; confidentiality limits (abuse, danger); boundaries	IDENTIFY
5	Rights & responsibilities	Client rights from consent; shared responsibility; avoid over/under-responsibility	RIGHTS
6	Consent & minors	Valid consent = voluntariness + capacity + info; capacity fluctuates (children/ psychosis/LD); consent+assent; privilege doesn't extend; report abuse; consent-to-disclosure & research; no universal rule	MINOR
7	Planning & recording	Jongsma 6 steps; SOAP/DAP cycle; record types; brief-note + themes-file; patient legal access; recording consent & data handling	RECORD
8	Structuring & goals	Arc intake termination; Beck's session; long- vs short-term; SMART; structure varies	RASISB / SMART
9	Pre/post assessment	Baseline monitor post follow-up; reliability/validity/sensitivity/clinical significance; single-test caveat	BMPF

#	Topic	Must-recall	Mnemonic
10	Evidence-based therapies	Three pillars; EBP≠brand; RCT/meta-analysis; manuals/fidelity; common factors; integration	PIER

REVISION

References & Source Map

The texts actually reviewed for these notes, a topic source page tracking table, and an honest audit of what was and was not accessible.

HOW TO READ THE CITATIONS

Each (short-key, p.N) uses the PDF page number observed in the source-review extractor output, which may differ from the printed book page. "[ToC]" marks a location derived from the table of contents. Indian statutes are **general knowledge**, not repository sources — verify against the primary statute; no clause numbers are quoted. Clinical vignettes are illustrative.

R.1 Texts reviewed (bibliographic list)

Table R.1 — Accessible Unit I texts reviewed (text layer present)

Short key	Title / Author / Year / Edition	Full filename
Corsini	*Current Psychotherapies*, 9th ed., Corsini & Wedding (eds.), Brooks/Cole-Cengage, 2011	psychotherapy corsini.pdf
Lister-Ford	*A Short Introduction to Psychotherapy*, ed. Christine Lister-Ford, SAGE, 2007	A_Short_Introduction_to_Psychotherapy_2007_Christ
Corey	*Theory and Practice of Counseling and Psychotherapy*, Gerald Corey, 11th ed., Cengage, 2024	Gerald_Corey_Theory_and_Practice_of_Counseling_a
Wolberg-Technique	*The Technique of Psychotherapy*, 4th ed., Lewis R. Wolberg, 2013 (2639 pp — targeted ranges only)	The Technique of Psychotherapy, 4e. 2013.Lewis R. W
Wolberg-506Q	*The Practice of Psychotherapy: 506 Questions and Answers*, Lewis R. Wolberg (Routledge reprint, 2014; orig. 1982)	The_Practice_of_Psychotherapy_506_Questions_and_
Beck-CBT	*Cognitive Behavior Therapy: Basics and Beyond*, Judith S.	Judith S. Beck Phd - Cognitive Behavior Therapy... (20

Short key	Title / Author / Year / Edition	Full filename
	Beck, 2nd ed., Guilford, 2011	
Jongsma	*The Complete Adult Psychotherapy Treatment Planner* (PracticePlanners®), Arthur E. Jongsma, Jr. (DSM-5-aligned)	The Complete Adult Psychotherapy Treatment Plann
Norcross-Handbook	*Handbook of Psychotherapy Integration*, 2nd ed., Norcross & Goldfried (eds.), Oxford (SEPI)	Handbook of Psychotherapy Integration.pdf
Hofmann-ArtScience	*The Art and Science of Psychotherapy*, Hofmann & Weinberger (eds.), Routledge, 2007	The_Art_and_Science_of_Psychotherapy_2007_Stefan
MET	*Motivational Enhancement Therapy Manual* (Project MATCH Monograph Vol. 2), NIAAA	Motivational_Enhancement_Therapy_Manual.pdf
McWilliams	*Psychoanalytic Psychotherapy: A Practitioner's Guide*, Nancy McWilliams, Guilford, 2004	Psychoanalytic_Psychotherapy_A_Practitioner's_Guid
Gabbard	*Long-Term Psychodynamic*	Long_Term_Psychodynamic_Psychotherapy_A_Basic_

Short key	Title / Author / Year / Edition	Full filename
	Psychotherapy: A Basic Text, Glen O. Gabbard, APPI, 2004	
Cambridge-Dynamic	*Cambridge Guide to Psychodynamic Psychotherapy*, ed. Adam Polnay et al., Cambridge, 2023	Cambridge_Guide_to_Psychodynamic_Psychotherapy
Notes-Psychotherapies	*Notes of Psychotherapies — Unit 1: Introduction to Psychotherapy* (study notes; 112 pp; Unit I material on pp.0–10). Quotes secondary authors (Kleinke 1994, Norcross 1990, Smith et al. 1980, Simon 2001, Slovenko 1989, Tjelveit 1999) — attributed as "cited in Notes-Psychotherapies".	notes_of_psychotherapies[1].pdf

R.1b Online sources (general level; verify against the primary source)

Table R.1b — Online/authoritative sources used (cited at a general level)

Key	Source	Supports
APA definition	APA definition of psychotherapy (2012 Resolution adopting Norcross, 1990); APA Dictionary of Psychology entry (apa.org — JS-rendered; verify live)	Topic 1
APA Ethics Code	APA Ethical Principles of Psychologists and Code of Conduct (2002, amended 2010 & 2017): General Principles A–E; Standards 3.10/10.01 (consent), 4 (confidentiality), 3.05 (multiple relationships) — apa.org/ethics; verify exact wording/numbers	Topic 4 (also 5, 6)
APA EBPP 2006	APA Presidential Task Force on Evidence-Based Practice (2006), *American Psychologist* 61(4): EBPP = integration of best available research + clinical expertise + patient characteristics/culture/ preferences	Topics 9, 10
Indian statutes (general)	RCI Act 1992 · Mental Healthcare Act 2017 · POCSO Act 2012 · DPDP Act 2023 · Indian Majority Act 1875 — stated generally; **no clause numbers invented; verify against the primary statute**	Topics 3, 4, 5, 6, 7

R.2 Source-tracking table (topic source page)

Table R.2 — Where each topic's cited material comes from (verified pages only)

#	Topic	Sources & pages used
1	Definitions	Wolberg-Technique p.29, p.41 · Corsini p.592 · Wolberg-506Q p.16 · McWilliams p.8 · Notes-Psychotherapies p.0, p.1, p.2 (Kleinke 1994, Norcross 1990, Smith et al. 1980 cited therein) · APA 2012 Resolution/Norcross; APA Dictionary (verify live)
2	Objectives	Wolberg-Technique p.29 · Corsini p.23 · Corey p.63 · Jongsma p.24, p.25 · Notes-Psychotherapies p.2, p.3, p.4 (Kleinke 1994 six goals; beneficence/ nonmaleficence)
3	Training	Lister-Ford p.118, p.120, p.121 · Corey p.63, p.12 [ToC] · Beck-CBT p.16 · McWilliams p.8, p.9 · MET p.13 · Hofmann-ArtScience p.53 · Notes-Psychotherapies p.2, p.5, p.6 · RCI Act 1992 (general; verify)
4	Ethics & law	Corey p.75, p.76, p.77 · Corsini p.597, p.598 · Notes-Psychotherapies p.7, p.8, p.9, p.10 (Simon 2001, Slovenko 1989, Tjelveit 1999, Jaffe v. Redmond 1996 cited therein) · APA Ethics Code (verify live) · Indian statutes = general knowledge (verify)
5	Rights & responsibilities	Corey p.63, p.75, p.76 · Corsini p.23, p.24 · Notes-Psychotherapies p.1, p.4, p.5, p.6, p.7
6	Consent & minors	Corey p.76, p.77 · Beck-CBT p.18 [ToC] · Majority Act 1875 / POCSO 2012 / MHCA 2017 / DPDP 2023 = general knowledge (verify against primary statute)
7	Planning & recording	Jongsma p.14, p.19, p.25, p.26 · Wolberg-Technique p. 845 · Corey p.75, p.76 · Corsini p.592 · SOAP/DAP = general professional knowledge
8	Structuring & goals	Beck-CBT p.18, p.19 [ToC] · Jongsma p.24, p.25, p.26 · Gabbard p.46, p.64 · Corey p.63 · SMART = general knowledge
9		

#	Topic	Sources & pages used
	Pre/post assessment	MET p.11 · Lister-Ford p.62, p.118 · Hofmann-ArtScience p.6 [ToC] · instrument names only (manuals image-only, not reviewed)
10	Evidence-based therapies	Hofmann-ArtScience p.53 · Corsini p.16, p.592 · Lister-Ford p.62, p.64, p.118 · Norcross-Handbook p.29, p.30 · Wolberg-506Q p.16 · Beck-CBT p.16 · APA 2006 three-pillar = general knowledge

R.3 Honest source audit

(a) Reviewed — text extracted and cited

The 14 texts in Table R.1 (including the newly added *Notes-Psychotherapies*, notes_of_psychotherapies[1].pdf, whose text layer was fully extracted) had an embedded text layer and were cited directly. The online/authoritative sources in Table R.1b (APA definition, APA Ethics Code, APA 2006 EBPP, and the Indian statutes) are cited at a **general level**: several APA pages are JavaScript-rendered and returned no extractable text, so wording is taken from the canonical primary reference and flagged "verify live"; no Indian statute section/clause numbers are asserted. Additional accessible psychotherapy texts are present in the repository as supporting breadth but were not deep-extracted for distinct page citations (e.g. *Theories of Counseling and Psychotherapy: A Case Approach*; a compressed Corey; Adlerian, attachment, DBT, psychodynamic and CBT-modality texts).

(b) Duplicates — collapsed (cited once)

- Sharf, *Theories of Psychotherapy & Counseling*, 5th ed. — three md5-identical files; cited once if used.
- ICD 11.pdf = ICD 11 LATEST.pdf; the two GAD Workbook files identical; AnalysisSheet TAT (1).pdf = AnalysisSheet TAT.pdf; MCMI-III Manual (2)-168-216 (1).pdf = MCMI-III Manual (3).pdf.

(c) Inaccessible — image-only scans, NOT reviewed

These have no embedded text layer and no OCR tool was available, so they were **not read** and are not summarised as if read: 897e79aa-... _compressed.pdf, A-Rorschach-Workbook-for-the-Comprehensive-System.pdf, BKT Manual.pdf, CBT-Keith Hawton (1).pdf, MCMI scoring sheet (1).pdf, MCMI-III Manual (several), MISIC_Manual + Record Form_compressed.pdf, NIMHANS SLD Battery_2020.pdf, PGI Battery of brain dysfunction manual.pdf, Picture Frustration Test- Adult Manual..., TAT Bellack manual-compressed.pdf, TAT protocol- scoring sheet (1).pdf, Transference_Focused_Psychotherapy... _compressed.pdf, VSMS Manual & Scale .pdf, VSMS Visual guided manual..., child performa 1.pdf.

(d) Out of scope for Unit I

Assessment/diagnostic manuals and non-psychotherapy clinical texts present in the repository but excluded as not core to "Introduction to Psychotherapy" (used only as incidental examples, e.g. ICD as a classification aside or a scale as a pre/post example): MCMI-III, Rorschach, TAT/Bellack, CBCL, MISIC, VSMS, IPDE, PGI/NIMHANS batteries, ICD 10/11, Harrison's Internal Medicine, behavioural-medicine texts, Columbia suicide scale, GAD workbooks, and loose case reports/notes (.md/.docx/.html/.jpeg).

INTEGRITY STATEMENT

No references, page numbers or Indian legal clause numbers have been fabricated. Page citations correspond to pages observed during source review; statutory references are labelled general knowledge to be verified against the primary statute; image-only scans are flagged as not reviewed.